

Patient Face Sheet

TRANSCEND HOSPICE 18 LLC

PATIENT INFORMATION	
Patient Name: ALVIN SCOTT	MR #: 87169
Admission Date: 10/28/2025	Admission #: 1 Current Status: Admitted (Hospice)
Primary Diagnosis (ICD10): I25.10 (Atherosclerotic heart disease of native coronary artery w/o ang pectrs) Related Conditions (ICD10): I10 (Essential (primary) hypertension), I70.90 (Unspecified atherosclerosis), N40.1 (Benign prostatic hyperplasia with lower urinary tract symp), R53.1 (Weakness), R52 (Pain, unspecified), R29.6 (Repeated falls), F41.9 (Anxiety disorder, unspecified), K59.00 (Constipation, unspecified), Z87.440 (Personal history of urinary (tract) infections), I50.32 (Chronic diastolic (congestive) heart failure), R19.7 (Diarrhea, unspecified), R60.9 (Edema, unspecified) Unrelated Conditions (ICD10): E11.22 (Type 2 diabetes mellitus w diabetic chronic kidney disease), E78.5 (Hyperlipidemia, unspecified)	
Level of Care: Routine Home Care	Disaster Code: Disaster Code 2
General Information:	
Birth Date: 5/10/1961	SSN: 467-19-3962
Race: Black or African American	Gender: Male
Ethnicity: African-American	
Marital Status: Single	Is DNR?: No
Other Information:	Primary Language: English
Allergies: NKDA	Religion: Christian
Funeral Home:	
Address:	
Patient Contact Information:	
Referral Information:	
Referral Source:	Called In By: Carla Rollerson
LOCATION INFORMATION	
Current Location:	
Location Type: Home	
Institution Name (facility patients):	
Address: 1101 Mountain View Street, Glenn Heights TX 75154-8649	
CAREGIVER INFORMATION	
Primary Caregiver	
Caregiver Name: Diamond Scott	Relationship to Patient: Daughter
Address:	
Mobile: (469) 769-0195	
PHYSICIAN INFORMATION	
Attending Physician Name:	NPI #:
Address:	
Hospice Physician Name: RESTREPO, DIEGO	NPI #: 1063664076
FINANCIAL INFORMATION	
Payer Name: Palmetto GBA	
Coverage ID: 1U86AA7PN76 Coverage Type: Medicare Part A	
Service(s): Hospice (Primary)	

Medication List

SCOTT, ALVIN (MR# 87169)

Generated On: 12/28/2025 8:57 PM

Allergies: NKDA

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
10/28/2025		24 HR glipizide 10 MG Extended Release Oral Tablet	10 mg	1tablet Extended Release Oral Tablet	Oral Pill	Once daily	DM	No	Active
10/28/2025		acetaminophen 325 MG / hydrocodone bitartrate 10 MG Oral Tablet [Norco]	325-10 mg	1 tablet Oral Tablet	Oral Pill	Four times daily	Pain	Yes	Active
10/28/2025		esomeprazole 20 MG Delayed Release Oral Capsule [Nexium]	20 mg	1 tablet Delayed Release Oral Capsule	po	Once daily	GERD	Yes	Active
10/28/2025		furosemide 20 MG Oral Tablet	20 mg	20mg Oral Tablet	po	Once daily	diuretics	Yes	Active
11/14/2025		melatonin 3 MG Oral Tablet	3 mg	1 tablet Oral Tablet	po	Give once at bedtime	insomnia	Yes	Active
11/14/2025		Promethazine	6.25/5ml	5 ml	Po	Every 4-6 hours prn	Cough / allergies	Yes	Active
11/14/2025	11/24/2025	amoxicillin 875 MG / clavulanate 125 MG Oral Tablet [Augmentin]	875-125 mg	1tablet Oral Tablet	Oral Pill	Twice daily for10 days	URI	Yes	Discontinued
Comfort Kit Medications:									
10/28/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/28/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/28/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/28/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/28/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/28/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/28/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/28/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL ASSESSMENT

SPIRITUAL ASSESSMENT

Date/Time in: 10/28/2025 1:00:00 PM
Date/Time out: 10/28/2025 1:30:00 PM
Location/address: Patient home

Response to Terminal Prognosis

- Acceptance
 - ☒ Patient
 - ☒ Representative

Pain Assessment

Pain rating: 0
Pain scale used: Non-Verbal Scale

Indicators of Spiritual Strengths

Comments related to spiritual strengths:	Chaplain affirmed the family's autonomy and provided reassurance that chaplain support remains available if future needs arise. The patient voiced appreciation for the call and expressed understanding that chaplain support can be accessed upon request.
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Indicators of Spiritual Concerns/Spiritual Distress

Teaching and other interventions provided during visit (include patient/family's response to teaching/interventions):	Chaplain reviewed new admission and attempted to initiate spiritual assessment. Patient resides in a group home where the owner has declined chaplain visitation for all residents. Direct contact with the patient was therefore not permitted. Staff report that the patient is alert, oriented, and calm, and has expressed appreciation for hospice nursing and comfort-focused care. Chaplain remains available by phone should the facility or patient request spiritual or emotional support in the future.
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Communication and Comments

Comments related to Plan of Care and overall impressions:	Chaplain reviewed new admission and attempted to initiate spiritual assessment. Patient resides in a group home where the owner has declined chaplain visitation for all residents. Direct contact with the patient was therefore not permitted. Staff report that the patient is alert, oriented, and calm, and has expressed appreciation for hospice nursing and comfort-focused care. Chaplain remains available by phone should the facility or patient request spiritual or emotional support in the future. Chaplain to maintain PRN status. Re-offer services monthly through coordination with the interdisciplinary team. Document restrictions in IDG note and continue attempts to engage facility leadership regarding access.
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Date: 10/31/2025

Electronically signed by: DAMIEN DAWSON M.DIV

Patient: ALVIN SCOTT
Medical Record: #87169

INITIAL COMPREHENSIVE ASSESSMENT (WITH HOPE)

COMPREHENSIVE ASSESSMENT

Date/Time In:	10/28/2025 1:52:00 PM
Date/Time Out:	10/28/2025 2:52:00 PM
Modality:	In-person visit
Describe onset of illness, progression of symptoms, functional status, response to treatments, etc: including any recent hospitalizations, ER visits, weight loss and infections:	The patient, a 64-year-old single, African-American male, has been admitted under routine home care hospice services with a primary diagnosis of atherosclerotic heart disease of native coronary artery without angina pectoris (I25.10). His admission from a community residential setting occurred on October 28, 2025. He is classified as Disaster Code 2 and holds a "Do Not Resuscitate" (DNR) status of No. The patient's comorbid conditions include essential hypertension (I10), unspecified atherosclerosis (I70.90), benign prostatic hyperplasia with lower urinary tract symptoms (N40.1), weakness (R53.1), unspecified pain (R52), repeated falls (R29.6), unspecified anxiety disorder (F41.9), unspecified constipation (K59.00), a personal history of urinary tract infections (Z87.440), and chronic diastolic congestive heart failure (I50.32). Additionally, unrelated conditions include type 2 diabetes mellitus with diabetic chronic kidney disease (E11.22) and unspecified hyperlipidemia (E78.5). The patient, who is a Christian and speaks English as his primary language, has an allergy to Vancomycin and does not currently have arrangements with a funeral home. He refuses supplemental oxygen and is no longer pursuing aggressive medical treatment. He voluntarily concurs with the current hospice plan of care. Since admission, the patient's general condition reflects a PPS score of 50 and an NYHA classification of stage 4. He has exhibited progressive symptoms, including increased shortness of breath upon ambulation and noticeable bilateral edema, which is being managed with Lasix and compression socks. Over the past six months, there has been a weight decline from 180 to 150 pounds, resulting in a Body Mass Index (BMI) of 22. The patient now consumes only 50% of a standard adult meal due to a reduced appetite. The patient is clinically assessed as a 3/6, indicating a moderate level of functional impairment. His care plan focuses on symptom management and maintaining quality of life within the limitations of his current health status.

CO-MORBIDITIES

What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?	
☒ Ischemic Heart Disease	
☒ Coronary Artery Disease	
☒ Diabetes	
☒ Generalized Muscle Weakness	

GENERAL INFORMATION

Race:	African-American
Religion:	Christian
Primary Language:	English
Does the patient or primary caregiver need an interpreter?	No
Is the patient or the patients spouse a veteran?	No

PSYCHOSOCIAL / SPIRITUAL / BEREAVEMENT ASSESSMENT

Patient/Responsible Party's Current Response to Diagnosis/Prognosis: (Check all that apply)

- Acceptance
- ☒ Patient

☒ Representative

• *Support Systems*

☒ Church

☒ Neighbors

☒ Friends

• *Spiritual Support*

Needs spiritual support? Yes

• *Psychosocial Support*

Needs psychosocial support? Yes

• *Patient/Family Goals*

☒ Patient and family will receive physical, spiritual, and emotional support

☒ Effective grieving

☒ Safe and comfortable dying

PAIN/COMFORT ASSESSMENT

Has patient experienced pain the past 7 days? Yes

Pain Assessment Scale utilized: Verbal (scale of 1-10)

Acceptable level of pain: 2

Best: 2

Worst: 5

☒ Generalized pain/undefined location

Pain Frequency: Constant

Pain Duration: Less than an hour

Bone/Somatic pain: Throbbing

Visceral pain: Aching

Neuropathic pain: Tingling

Non-Verbal Signs of Pain

☒ Moaning/crying

☒ Grimacing

☒ Guarding

What makes the pain better?

☒ Relaxation techniques

☒ Rest

☒ Medication

What causes pain or makes it worse?

☒ Standing

☒ Sitting

What other symptoms are associated with the pain?

☒ Decreased mobility

- ☒ Poor concentration
- ☒ Decreased socialization
- ☒ Decreased appetite

Current treatments for pain:

- ☒ Opioids

Patient is on a scheduled opioid, currently taking a PRN opioid or has been instructed to begin taking an opioid: Yes

Specify the scheduled opioids the patient is currently taking (leave blank if n/a): Morphine

Specify the PRN opioids the patient is taking (leave blank if n/a): Morphine

Patient is on a bowel regime: Yes

Date bowel regime initiated: 10/28/2025

Describe bowel regime: Senna

How much breakthrough pain medication has patient taken in the past 24 hours? 3

Is current pain regime effective? Yes

Patient/family pain management goals Relieve pain/discomfort as much as possible even if it will cause increased sedation

Patient/caregiver educated on pain regime and side effects of pain medications: Yes, and verbalized understanding

Patient educated on non-medication treatments of pain? Yes

☒ Massage

Non-medication treatments were initiated: Yes

☒ Breathing exercises

HEAD TO TOE ASSESSMENT

Vital Signs

Type of temperature taken: Temporal:

Temporal temperature: 98

Respiratory rate (respirations per minute): 22

Respiratory pattern: Regular

SaO2 94

Pulse Type: Apical

Heart rate: 100

Blood pressure type: Sitting

Systolic: 150

Diastolic: 89

Height: 5'8"

Weight: 140

Arm circumference taken: Left

Arm circumference: 22

Nutrition

- ☒ Diet Orders (specify):
- ☒ General (as tolerated)

Average percentage of meals consumed: 50

Patient Feeding: Patient able to feed self independently

Neurological Status

Orientation

- ☒ Self
- ☒ Location
- ☒ Time

Behavior

- ☒ Calm
- ☒ Agitated
- ☒ Restlessness
- ☒ Forgetful

Energy

- ☒ Fatigue/Weakness
- ☒ Increased sleep

Number of hours spent sleeping in a day: 14

Responsiveness

Responsiveness: Verbal stimuli

Intellect

Intellect: No deficit

Speech

- ☒ Clear

Neuromusculoskeletal Status

- ☒ Normal Balance, coordination and mobility

Muscle Tone

- ☒ Muscle wasting

Functional Assessment

Functional Assessment Score: 5

Gastrointestinal Status

Last BM Today

Bowel Sounds positive in all 4 Quadrants

- ☒ Normal
 - ☒ RUQ
 - ☒ RLQ
 - ☒ LUQ
 - ☒ LLQ

- ☒ Incontinence

Genitourinary Status

- ☒ WNL
- ☒ Urinary incontinence

Integumentary Status

- ☒ Skin intact

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort:	1
Moisture: degree to which skin is exposed to moisture:	1
Activity: degree of physical activity:	1
Mobility: ability to change and control body position:	1
Nutrition: usual food intake pattern:	1
Friction and shear:	1
Total score:	6

CARDIOPULMONARY ASSESSMENT

Was patient screened for shortness of breath	1. Yes
Date of first screening for shortness of breath	10/28/2025
Did screening indicate the patient had shortness of breath	1. Yes
Was treatment for shortness of breath initiated	2. Yes
Date treatment for shortness of breath initiated	10/28/2025

Respiratory Treatments for Shortness of Breath:

- ☒ Respiratory Status WNL

- ☒ Oxygen

Liters: 2
Via: NC
Frequency: As needed

- ☒ Dyspnea

- ☒ At rest

- ☒ Decreased

- ☒ RUL
- ☒ RLL
- ☒ RML
- ☒ LLL
- ☒ LUL

- ☒ Capillary Refill (Fingers)

- ☒ More than 3 seconds

- ☒ Edema

Location: Bilateral
Type: Non-pitting

NY Heart Association Scale: Class IV (Severe)

FUNCTIONAL STATUS

Walking/Locomotion Independent
Bed Mobility; Independent
Transfers: Independent
Dressing: Needs Assistance
Eating: Independent
Toilet use: Needs Assistance
Personal hygiene: Needs Assistance
Bathing Needs Assistance
Palliative performance scale (PPS): 50% - Unable to do any Work

Fall Risk Assessment (NHPCO Guidelines)

• History of a fall within the last 6 months

☒ 3: More than 2 falls

• Mobility

☒ 2: Has a gait and/or balance problem

• Elimination

☒ 3: Incontinent all or most of the time

• Mental Status

☒ 4: Uncooperative or unable to follow directions

• Use of medication

☒ 1: Narcotic

• Medical conditions which could impair the ability to ambulate

- ☒ 1: Chronic debilitating illness
- ☒ 1: Weakness or paralysis of the extremities
- ☒ 1: Anti-hypertensives

Total: 10
☒ 6-10 points = Moderate risk

HOME SAFETY ASSESSMENT

☒ No safety problems identified

ASSESSMENT OF PATIENT/CAREGIVER TO SAFELY ADMINISTER MEDICATIONS

- ☒ Patient/ Caregiver/POA educated on medication regime and side effects
- ☒ Patient/Caregiver/POA verbalized understanding of Medication teaching provided

HOPE Admission Assessment

Based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less? 0. No
Living Arrangements 2. With others in the home (family, friends, paid caregiver)

Availability of Assistance

1. Around-the-clock (24 hours a day with few exceptions)

Interpreter requested

0. No

Preferences

Was patient/responsible party asked about CPR1. Yes

Date asked about CPR 10/28/2025

Was patient/responsible party asked about treatments other than CPR1. Yes

Date asked about treatment other than CPR 10/28/2025

Was patient/responsible party asked about hospitalization1. Yes

Date asked about hospitalization 10/28/2025

Was patient/responsible party asked about spiritual/existential concerns1. Yes

Date asked about spiritual/existential concerns 10/28/2025

Active Diagnoses

Principal Diagnosis

07. Heart Failure

Comorbidities and Co-existing Conditions

☒ Diabetes Mellitus (DM)

☒ Other Medical Condition

Pain

Was patient screened for pain1. Yes

Date of first screening for pain10/28/2025

Patient's pain severity was2. Moderate

Type of standardized pain tool used2. Verbal descriptor

Is pain an active problem for the patient?1. Yes

Was comprehensive pain assessment done1. Yes

Date of comprehensive pain assessment10/28/2025

Comprehensive pain assessment included:

☒ Location

☒ Severity

☒ Character

☒ Frequency

☒ Duration

☒ What relieves/worsens pain

☒ Effect on function or quality of life

Neuropathic Pain

1. Yes

Symptom Impact

Was a symptom impact screening completed?1. Yes

Date of symptom impact screening10/28/2025

Pain2. Moderate

Shortness of breath2. Moderate

Anxiety1. Slight

Nausea0. Not at all

Vomiting0. Not at all

Diarrhea0. Not at all

Constipation 0. Not at all
Agitation 1. Slight

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 1. Yes
Date scheduled opioid initiated or continued 10/28/2025
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 10/28/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 10/28/2025

TEACHING/EDUCATION PROVIDED

Education Provided To:

☒ Patient

Topics

- ☒ Palliative care, hospice philosophy, hospice benefit
- ☒ Scope of services, role of team members
- ☒ Call us First Program
- ☒ Diagnosis
- ☒ Prognosis
- ☒ Disease process
- ☒ Safe & effective use of equipment & supplies
- ☒ Dying process, signs & symptoms of approaching death
- ☒ Covered medications, supplies, equipment
- ☒ How to re-order medications and supplies
- ☒ Medication usage, side effects, drug interactions
- ☒ Basic home safety, emergency preparedness
 - ☒ Provided written personalized emergency preparedness plan
- ☒ Infection control, disposal of hazardous waste, universal precautions
- ☒ Skin care, wound care, dressing changes
- ☒ Energy Conservation techniques
- ☒ Plan of care established with primary care giver
- ☒ Symptom management
- ☒ Catheter care
- ☒ Bowel regimen
- ☒ Pain management
- ☒ What to do at the time of death
- ☒ Anticipatory grief, grief, bereavement

- ☒ Funeral planning
- ☒ Advance directives
- ☒ Community resources
- ☒ Signs & symptoms of infection
- ☒ Discharge planning

COMMUNICATION WITH OTHER TEAM MEMBERS

- ☒ Hospice Medical Director
- ☒ Clinical Director/PCC
- ☒ Assigned RN Case Manager
- ☒ Assigned LPN
- ☒ Social Worker
- ☒ Volunteer Coordinator
- ☒ Bereavement Coordinator
- ☒ Spiritual Counselor

Date: 10/28/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

BEREAVEMENT RISK ASSESSMENT

Date/Time In10/30/2025 10:49:00 AM

Date/Time Out10/30/2025 11:00:00 AM

Risk Assessment Grid

Caregiver	Diamond Scott (primary - Daughter)
1: Possible alcohol/substance abuse	☐
1: Neglect of appearance	☐
1: History of mental or emotional health concerns	☐
1: History of marked dependence on patient	☐
1: History of suicide ideation or intent	☐
1: Age less than 18	☐
1: Absence of meaningful spiritual support	☐
1: Death means loss of constant companion or emotional support	☐
1: Death will result in loss of financial provisions/financial support	☐
1: Death means change or loss of role or function	☐
1: Unable to share feelings	☐
1: Difficulty making decisions	☐
1: Ambivalent relationship with the patient	☐
1: Death means loss of meaning/purpose in life	☐
1: Communication difficulties	☐
1: Cultural issues present	☐
1: Inflexible thinking	☐
1: Dependent family members/children living in patient's home	☐
1: Unfinished business with patient	☐
1: Sense of hopelessness	☐
1: Signs of spiritual distress	☐
1: Estranged or isolated from support system	☐
1: Inadequate or negative coping skills	☐
1: Concurrent life crisis	☐
1: Multiple or recent losses	☐
1: Unprepared for death/strong denial prior to death	☐
1: Difficulty coping with past losses	☐
1: Unresolved grief from prior losses	☐
1: Disenfranchised grief	☐
1: History of family violence	☐
1: Family or marital conflict	☐
Risk Total:	
Comments	Chaplain reached out to patient's daughter to complete assessments. On 10/28, 31. She has not responded to date. Lack of engagement may indicate limited support or understanding of hospice. Plan Continue outreach and document attempts. Notify bereavement coordinator at time of death for potential follow-up screening. Risk assessed as low

Patient: ALVIN SCOTT
Medical Record: #87169

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 10/28/2025
Benefit Period: First 90 Days
Certification Period - From: 10/28/2025
Certification Period - To: 1/25/2026
Primary Diagnosis: Athscl heart disease of native coronary artery w/o ang pctrs
ICD-10 Code: I25.10

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Pt is a 64-year-old male with a primary diagnosis of atherosclerotic heart disease of native coronary artery without angina pectoris . The patient's comorbid conditions include essential hypertension unspecified atherosclerosis , benign prostatic hyperplasia with lower urinary tract symptoms , weakness unspecified pain , repeated falls unspecified anxiety disorder unspecified constipation , a personal history of urinary tract infections , and chronic diastolic congestive heart failure . Patient admitted to hospice with the following declines in the past 3 to 6 month. Pt with PPS score of 50 and an NYHA classification of stage 4. He has exhibited progressive symptoms, including increased shortness of breath upon ambulation and noticeable bilateral edema. Over the past six months, there has been a weight decline from 180 to 150 pounds, resulting in a Body Mass Index (BMI) of 22. The patient now consumes only 50% of a standard adult meal due to a reduced appetite. Patient needing assistance with 3 out of 6 ADLs. Patient hospice appropriate life expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD
Date: 10/28/2025

Patient: ALVIN SCOTT
Medical Record: #87169

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 10/28/2025
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Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD
Date: 10/28/2025

Patient: ALVIN SCOTT
Medical Record: #87169

VERBAL PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Verbal Physician Initial Certification of Terminal Illness

Start of Care Date:

10/28/2025

Benefit Period:

First 90 Days

Certification Period - From:

10/28/2025

Certification Period - To:

1/25/2026

Primary Diagnosis:

Athscl heart disease of native coronary artery w/o ang pctrs

ICD-10 Code:

I25.10

Hospice Physician Initial Certification of Terminal Illness

Verbal Certification:

On

10/28/2025

Dr.

DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

10/28/2025

Physician Signature / Date:

Attending Physician Initial Certification of Terminal Illness

Verbal Certification:

On

10/28/2025

Dr.

DIEGO F RESTREPO

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date:

10/28/2025

Physician Signature / Date:

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Beneficiary: Scott Alvin D
DOD:

Medicare ID: 1U86AA7PN76

Sex: Male

DOB: 05/10/1961

Beneficiary Information

Beneficiary Last Name:*

Scott

Beneficiary First Name:**

Alvin

Medicare ID: *

1U86AA7PN76

Beneficiary Birth Date:**

05/10/1961

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

Eligibility

Part A Eligibility

Beneficiary insured due to disability

Effective Date:

01/01/2005

Termination Date:

Part B Eligibility

Beneficiary insured due to disability

Effective Date:

01/01/2019

Termination Date:

Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

Inactive Periods

Effective Date:

Termination Date:

Reason:

Beneficiary Address

Address Line 1:

1101 Mountain View St

Address Line 2:

City:

Glenn Heights

State:

TX

Zip:

75154-8649

End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

Additional Information**MBI End Date**

End Date:

Audiology Screening

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
92550	07/01/2023		257	20%
92552	07/01/2023		257	20%
92553	07/01/2023		257	20%
92555	07/01/2023		257	20%
92556	07/01/2023		257	20%
92557	07/01/2023		257	20%
92562	07/01/2023		257	20%
92563	07/01/2023		257	20%
92565	07/01/2023		257	20%
92567	07/01/2023		257	20%
92568	07/01/2023		257	20%
92570	07/01/2023		257	20%
92571	07/01/2023		257	20%
92572	07/01/2023		257	20%
92575	07/01/2023		257	20%
92576	07/01/2023		257	20%
92577	07/01/2023		257	20%
92579	07/01/2023		257	20%
92582	07/01/2023		257	20%

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
92583	07/01/2023		257	20%
92584	07/01/2023		257	20%
92587	07/01/2023	07/01/2023	257	20%
92588	07/01/2023	07/01/2023	257	20%
92601	07/01/2023		257	20%
92602	07/01/2023		257	20%
92603	07/01/2023		257	20%
92604	07/01/2023		257	20%
92620	07/01/2023		257	20%
92621	07/01/2023		257	20%
92622	07/01/2023		257	20%
92623	07/01/2023		257	20%
92625	07/01/2023		257	20%
92626	07/01/2023		257	20%
92627	07/01/2023		257	20%
92640	07/01/2023		257	20%
92651	07/01/2023		257	20%
92652	07/01/2023		257	20%
92653	07/01/2023		257	20%

MDPP Coverage

Start Date:

End Date:

MDPP Inactive Periods

Start Date:

End Date:

MDPP Deductible

Start Date:

End Date:

Deductible Amount:

MDPP Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

MDPP Usage Information

HCPCS Code

Billing NPI

Date Of Service

Deductibles/Caps

Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20

Eligibility begins:

01/21/2020

Professional Sessions Remaining

20

Part B Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:
\$257

Part B Remaining Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$95.29

Co-insurance Details

Start Date:
01/01/2025
End Date:
12/31/2025
Co-insurance Amount:
20%

Blood Deductible

Calendar Year:
2025
Number Of Units Remaining:
3

Occupational Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Physical And Speech Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Pulmonary Rehabilitation Services

Calendar Year:

2025

Professional Sessions Remaining:

72

Technical Sessions Remaining:

72

Cardiac Rehabilitation Services

Calendar Year:

2025

Professional Sessions Used:

0

Technical Sessions Used:

0

Intensive Cardiac Rehabilitation Services

Calendar Year:

2025

Professional Sessions Used:

0

Technical Sessions Used:

0

Part B Free Services List of STC Codes (listSTCCodes.do)

STC Codes:

42,5,67,80,AJ,CO

Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Mental Health Co-insurance (if different from the plan level Co-insurance) List of STC Codes (listSTCCodes.do)

STC Codes:

Value:

Start Date:

End Date:

Preventive

COVID

COVID-19 Immunization

Immunization Date

HCPC Code

Rendering Medicare NPI

No data available

Pneumococcal Vaccine (PPV)

Pneumococcal Vaccine

Date of Service

HCPC Code

Rendering Medicare NPI

12/14/2011

90732

1275592131

Flu

Flu Vaccination

Vaccination Date

HCPC Code

Rendering Medicare NPI

No data available

Cognitive Assessment and Care Plan Services

Cognitive Assessment and Care Plan Services

Date of Service

HCPC Code

Rendering Medicare NPI

No data available

Smoking Cessation

Number of Sessions in Benefit Period:

8

Initial Session Date:

Benefit Period Sessions Remaining:

8

Preventive

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
71271	01/01/2021	01/01/2021	0	0%
76706	07/01/2007	07/01/2007	0	0%
76977	01/01/2019	01/01/2019	0	0%
77067	01/01/2019	01/01/2019	0	0%
77078	01/01/2019	01/01/2019	0	0%
77080	01/01/2019	01/01/2019	0	0%
77081	01/01/2019	01/01/2019	0	0%
80061	01/01/2019	08/01/2016	0	0%
81528		01/01/2019	0	0%
82270	01/01/2019	01/01/2019	0	0%
82465	01/01/2019	08/01/2016	0	0%
82947	01/01/2019	01/01/2019	0	0%
82950	01/01/2019	01/01/2019	0	0%
82951	01/01/2019	01/01/2019	0	0%
83036	01/01/2024	01/01/2024	0	0%
83718	01/01/2019	08/01/2016	0	0%
84478	01/01/2019	08/01/2016	0	0%
G0476		07/09/2015	0	0%
G0475		01/01/2019	0	0%
G0473	01/01/2019	01/01/2019	0	0%
G0118	01/01/2019	01/01/2019	257	20%
G0117	01/01/2019	01/01/2019	257	20%

HCPCS Code	Next Professional Date	Next Technical Date	Remaining Deductible	Co-insurance
G0105	01/01/2019	01/01/2019	0	0%
G0104	01/01/2019	01/01/2019	0	0%
G0103	02/01/2022	01/01/2019	0	0%
G0102	01/01/2019	01/01/2019	257	20%
G0101	01/01/2019	01/01/2019	0	0%
G0447	01/01/2019	01/01/2019	0	0%
G0446	01/01/2019	01/01/2019	0	0%
G0445	01/01/2019	01/01/2019	0	0%
G0444	01/01/2019	01/01/2019	0	0%
G0442	01/01/2019		0	0%
G0148	01/01/2019	01/01/2019	0	0%
G0147	01/01/2019	01/01/2019	0	0%
G0145	01/01/2019	01/01/2019	0	0%
G0144	01/01/2019	01/01/2019	0	0%
G0143	01/01/2019	01/01/2019	0	0%
G0439	04/01/2023		0	0%
G0438	04/01/2023		0	0%
P3000	01/01/2019	01/01/2019	0	0%
G0130	01/01/2019	01/01/2019	0	0%
G0328	01/01/2019	01/01/2019	0	0%
G0327		07/01/2021	0	0%
G0123	01/01/2019	01/01/2019	0	0%
G0121	01/01/2019	01/01/2019	0	0%
Q0091	01/01/2019	01/01/2019	0	0%

Hepatitis Screening

Date Of Service	HCPCS Code	Provider or Rendering NPI	Deductible	Co-insurance
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Plan Coverage

Medicare Advantage

Plan Type:

Preferred Provider Organization(PPO)

Enrollment Date:

08/01/2024

Disenrollment Date:**Contract Number:**

H5216

Contract Name:

HUMANA INSURANCE COMPANY

Plan Number:

351

Plan Name:

Humana USAA Honor Giveback with Rx

Address Line 1:

1100 Employers Boulevard

Phone Number:

8004486262

Address Line 2:**City:**

De Pere

State:

WI

Zip Code:

54115

Website

www.humana.com/medicare

Bill Code :

C

VBID Model Hospice Benefit Component Links

[Hospice Benefit Component participating plans \(https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-participating-plans\)](https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-participating-plans)

[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model \(https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-billing-payment\)](https://innovation.cms.gov/innovation-models/vbid-hospice-benefit-billing-payment)

Medicare Part D

Enrollment Date:

08/01/2024

Disenrollment Date:

Contract Number:

H5216

Contract Name:

HUMANA INSURANCE COMPANY

Plan Number:

351

Plan Name:

Humana USAA Honor Giveback with Rx

Address Line 1:

1100 Employers Boulevard

Phone Number:

8004486262

Address Line 2:

City:

De Pere

State:

WI

Zip Code:

54115

Website

www.humana.com/medicare

Drug Plan :

OT

Enrollment

Y

Medicare Secondary Payer

Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

Hospice/HomeHealth

Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

02/17/2020

HH Recertification Start Date(s):

05/31/2022

04/01/2022

01/31/2022

12/02/2021

10/03/2021

08/04/2021

04/07/2021

04/17/2020

Hospice**Hospice Episodes:**

Effective Date	Termination Date	Start Date (DOEBA)	End Date (DOLBA)	Hospice Days Used	Provider Number	Provider Number Type
---------------------------	-----------------------------	-------------------------------	-----------------------------	------------------------------	----------------------------	-------------------------------------

Notices of Election (NOE):

Election Date	Election Receipt Date	Provider Number	Provider Number Type	Revocation Code	Election Revocation Date
--------------------------	----------------------------------	----------------------------	---------------------------------	----------------------------	-------------------------------------

Part A Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$1676

Part A Base Deductible Remaining

Start Date:

01/01/2025

End Date:

12/31/2025

Remaining Deductible:

\$1676

Deductible Remaining By Spell

DOEBA Date

DOLBA Date

Deductible Amt

No data to display

Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates

No data to display

Inpatient Base Summary

Start Date	End Date	Full Days Allowed	Full Copayment Days Allowed	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full Inpatient Days	Full Inpatient Copay Days	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

SNF Base Summary

Start Date	End Date	SNF Full Days Allowed	SNF Full Copayment Days Allowed	SNF Copay Amount
01/01/2025	12/31/2025	20	80	\$209.5

SNF Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full SNF Days	Full SNF Copay Days	SNF Copay Amount
01/01/2025	12/31/2025	20	80	209.5

Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

Part A Free Services List of STC Codes (listSTCCodes.do)

STC Codes:

42,45

Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

Medicaid Enrollment

State:

Enrollment Date:

Termination Date:

Beneficiary: Scott Alvin D
DOD:

Medicare ID: 1U86AA7PN76

Sex: Male

DOB: 05/10/1961

Beneficiary Information

Beneficiary Last Name:*

Beneficiary First Name:**

Medicare ID: *

Beneficiary Birth Date:**

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

Eligibility

Part A Eligibility

Beneficiary insured due to disability

Effective Date:

01/01/2005

Termination Date:

Part B Eligibility

Beneficiary insured due to disability

Effective Date:

01/01/2019

Termination Date:

Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

Inactive Periods

Effective Date:

Termination Date:

Reason:

Beneficiary Address

Address Line 1:
1101 Mountain View St
Address Line 2:

City:
Glenn Heights

State:

TX

Zip:
75154-8649

End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

Additional Information

MBI End Date

End Date:

Audiology Screening

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

92550	07/01/2023		257	20%
92552	07/01/2023		257	20%
92553	07/01/2023		257	20%
92555	07/01/2023		257	20%
92556	07/01/2023		257	20%
92557	07/01/2023		257	20%
92562	07/01/2023		257	20%
92563	07/01/2023		257	20%
92565	07/01/2023		257	20%
92567	07/01/2023		257	20%
92568	07/01/2023		257	20%
92570	07/01/2023		257	20%
92571	07/01/2023		257	20%
92572	07/01/2023		257	20%
92575	07/01/2023		257	20%
92576	07/01/2023		257	20%
92577	07/01/2023		257	20%

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

92579	07/01/2023		257	20%
92582	07/01/2023		257	20%
92583	07/01/2023		257	20%
92584	07/01/2023		257	20%
92587	07/01/2023	07/01/2023	257	20%
92588	07/01/2023	07/01/2023	257	20%
92601	07/01/2023		257	20%
92602	07/01/2023		257	20%
92603	07/01/2023		257	20%
92604	07/01/2023		257	20%
92620	07/01/2023		257	20%
92621	07/01/2023		257	20%
92622	07/01/2023		257	20%
92623	07/01/2023		257	20%
92625	07/01/2023		257	20%
92626	07/01/2023		257	20%
92627	07/01/2023		257	20%
92640	07/01/2023		257	20%
92651	07/01/2023		257	20%
92652	07/01/2023		257	20%
92653	07/01/2023		257	20%

MDPP Coverage

Start Date:

End Date:

MDPP Inactive Periods

Start Date:

End Date:

MDPP Deductible

Start Date:

End Date:

Deductible Amount:

MDPP Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

MDPP Usage Information**HCPCS Code Billing NPI Date Of Service**

Deductibles/Caps

Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20

Eligibility begins:

01/21/2020

Professional Sessions Remaining

20

Part B Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$257

Part B Remaining Deductible

Start Date:

01/01/2025

End Date:

12/31/2025

Deductible Amount:

\$0

Co-insurance Details

Start Date:

01/01/2025

End Date:

12/31/2025

Co-insurance Amount:

20%

Blood Deductible

Calendar Year:

2025

Number Of Units Remaining:

3

Occupational Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Physical And Speech Therapy Cap

Calendar Year:
2025
Amount Used:
\$0

Pulmonary Rehabilitation Services

Calendar Year:
2025
Professional Sessions Remaining:
72
Technical Sessions Remaining:
72

Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Intensive Cardiac Rehabilitation Services

Calendar Year:
2025
Professional Sessions Used:
0
Technical Sessions Used:
0

Part B Free Services List of STC Codes

STC Codes:
42,5,67,80,AJ,CO
Value:
0%
Start Date:
01/01/2025
End Date:
12/31/2025

Mental Health Co-insurance (if different from the plan level Co-insurance) STC Codes

[List of](#)

STC Codes:

Value:

Start Date:

End Date:

Preventive

COVID

COVID-19 Immunization

Immunization Date	HCPC Code	Rendering Medicare NPI
No data available		

Pneumococcal Vaccine (PPV)

Pneumococcal Vaccine

Date of Service	HCPC Code	Rendering Medicare NPI
12/14/2011	90732	1275592131

Flu

Flu Vaccination

Vaccination Date	HCPC Code	Rendering Medicare NPI
No data available		

Cognitive Assessment and Care Plan Services

Cognitive Assessment and Care Plan Services

Date of Service	HCPC Code	Rendering Medicare NPI
No data available		

Smoking Cessation

Number of Sessions in Benefit Period:

8

Initial Session Date:

Benefit Period Sessions Remaining:

8

Preventive

HCPCS Code	Next Professional Date	Next Technical Date	Remaining	Deductible Co-insurance
71271	01/01/2021	01/01/2021	0	0%
74263	01/01/2025	01/01/2025	0	0%
76706	07/01/2007	07/01/2007	0	0%
76977	01/01/2019	01/01/2019	0	0%

HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

77078	01/01/2019	01/01/2019	0	0%
77080	01/01/2019	01/01/2019	0	0%
77081	01/01/2019	01/01/2019	0	0%
80061	01/01/2019	08/01/2016	0	0%
81528		01/01/2019	0	0%
82270	01/01/2019	01/01/2019	0	0%
82465	01/01/2019	08/01/2016	0	0%
82947	01/01/2019	01/01/2019	0	0%
82950	01/01/2019	01/01/2019	0	0%
82951	01/01/2019	01/01/2019	0	0%
83036	01/01/2024	01/01/2024	0	0%
83718	01/01/2019	08/01/2016	0	0%
84478	01/01/2019	08/01/2016	0	0%
G0473	01/01/2019	01/01/2019	0	0%
G0118	01/01/2019	01/01/2019	257	20%
G0117	01/01/2019	01/01/2019	257	20%
G0105	01/01/2019	01/01/2019	0	0%
G0104	01/01/2019	01/01/2019	0	0%
G0103	02/01/2022	01/01/2019	0	0%
G0102	01/01/2019	01/01/2019	257	20%
G0447	01/01/2019	01/01/2019	0	0%
G0446	01/01/2019	01/01/2019	0	0%
G0445	01/01/2019	01/01/2019	0	0%
G0444	01/01/2019	01/01/2019	0	0%
G0442	01/01/2019		0	0%
G0439	04/01/2023		0	0%
G0438	04/01/2023		0	0%
0464U		01/01/2019	0	0%
G0130	01/01/2019	01/01/2019	0	0%
G0328	01/01/2019	01/01/2019	0	0%
G0327		07/01/2021	0	0%
G0121	01/01/2019	01/01/2019	0	0%

Hepatitis Screening**Date Of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance****HIV/PrEP - Pre-Exposure Prophylaxis for HIV Prevention****Date of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance**

No data available

Plan Coverage**Medicare Advantage**

Plan Type:

Preferred Provider Organization(PPO)

Enrollment Date:
08/01/2024
Disenrollment Date:
Contract Number:
H5216
Contract Name:
HUMANA INSURANCE COMPANY
Plan Number:
351
Plan Name:
Humana USAA Honor Giveback with Rx
Address Line 1:
1100 Employers Boulevard
Phone Number:
8004486262
Address Line 2:
City:
De Pere
State:
WI
Zip Code:
54115
Website
www.humana.com/medicare
Bill Code :
C

VBID Model Hospice Benefit Component Links

[Hospice Benefit Component participating plans](#)

[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model](#)

Medicare Part D

Enrollment Date:
08/01/2024
Disenrollment Date:
Contract Number:
H5216
Contract Name:
HUMANA INSURANCE COMPANY
Plan Number:
351
Plan Name:
Humana USAA Honor Giveback with Rx
Address Line 1:
1100 Employers Boulevard
Phone Number:
8004486262

Address Line 2:

City:

De Pere

State:

WI

Zip Code:

54115

Website

www.humana.com/medicare

Drug Plan :

OT

Enrollment

Y

Medicare Secondary Payer

Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

Hospice/HomeHealth

Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

HH Recertification Start Date(s):

05/31/2022

04/01/2022

01/31/2022

12/02/2021

10/03/2021

08/04/2021

04/07/2021

Hospice

Hospice Episodes:

Effective Date	Termination Date	Start Date (DOEBA)	End Date (DOLBA)	Hospice Days Used	Provider Number	Provider Number Type
----------------	------------------	--------------------	------------------	-------------------	-----------------	----------------------

Notices of Election (NOE):

Date	Election Receipt Date	Provider Number	Provider Number Type	Revocation Code	Election Revocation Date
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Part A Deductible

Start Date:
01/01/2025
End Date:
12/31/2025
Deductible Amount:
\$1676

Part A Base Deductible Remaining

Start Date:
01/01/2025
End Date:
12/31/2025
Remaining Deductible:
\$1676

Deductible Remaining By Spell

DOEBA Date DOLBA Date Deductible Amt
No data to display

Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates

No data to display

Inpatient Base Summary

Start Date	End Date	Full Days Allowed	Full Copayment Days Allowed	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full Inpatient Days	Full Inpatient Copay Days	Copay Amount
01/01/2025	12/31/2025	60	30	\$419

SNF Base Summary

Start Date	End Date	SNF Full Days Allowed	SNF Full Copayment Days Allowed	SNF Copay Amount
01/01/2025	12/31/2025	20	80	\$209.5

SNF Days Remaining (May Also Include Spells)

DOEBA Date	DOLBA Date	Full SNF Days	Full SNF Copay Days	SNF Copay Amount
01/01/2025	12/31/2025	20	80	209.5

Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

Part A Free Services List of STC Codes

STC Codes:

42,45

Value:

0%

Start Date:

01/01/2025

End Date:

12/31/2025

Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

Medicaid Enrollment

State:

Enrollment Date:

Termination Date:

Part A Deductible

Start Date:

End Date:

Deductible Amount:

Inpatient Base Summary

Start Date:
End Date:
Full Days Allowed:
Full Copay Days Allowed:
Copay Amount:

Inpatient Days Remaining (May Also Include Spells)

DOEBA Date:
DOLBA Date:
Full Inpatient Days:
Full Inpatient Copay Days:
Copay Amount:

SNF Base Summary

Start Date:
End Date:
SNF Full Days Allowed:
SNF Full Copay Days:
Copay Amount:

SNF Days Remaining (May Also Include Spells)

DOEBA Date:
DOLBA Date:
Full SNF Days:
Full SNF Copay Days:
Copay Amount:

Lifetime Reserve

Lifetime Days Allowed:
60
Lifetime Days Remaining :
60
Start Date:
End Date:
Copolyment Amount:

Part B Deductible

Start Date:
End Date:
Deductible Amount:

Part B Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:



—HOSPICE—

REFERRAL/INTAKE FORM

Referral Date:	10-21-2025	Form Completed By:	Tina King
Referral Obtained by:	Carla Rollerson	Referral Source:	Carla Rollerson

PATIENT INFORMATION SUMMARY

Patient Name (Match Medicare Card):	Alvin Scott	Patient Address:	Glenn Heights Tx 75154
DOB:	5/10/1961	Facility & Room #:	1101 Mountain View St
SS#:	467-19-3962	County of Residence:	
Age:		Phone:	
Height/Weight:		Marital Status:	
Gender:		Religion:	
Race:		Allergies:	
Diagnosis:		Preferred Language:	
Name of Hospital (for Medical Records Request):			

COVERAGE & PLAN DETAILS

Medicare MBI:		Medicaid Effective Date:	
MBI Effective Date:		Medicare MBI:	
Other Insurance #:		Currently on Hospice Services?	
Insurance Phone/Address:		Currently on HH?	

CARE DIRECTIVES & SERVICE RECOGNITION

Funeral Home:		Veteran: Y/N	
Funeral Phone:		Military Branch:	
DNR: Y/N		Enrolled in VA: Y/N	
Attending MD:		Service Era:	
Attending Phone #:			
Continue as Attending: Y/N:			

DESIGNATED REPRESENTATIVES

Primary CG/POA Name:		Phone #:	
Relationship to Patient:		Alt. Phone #:	
Address:		Email Address:	
Emergency Contact Name:		Phone #:	
Relationship to Patient:		Alt. Phone #:	
Address:		Email Address:	

PT/FAMILY READY TO PROCEED WITH ADMISSION: ☐ YES ☐ NO IF NO, THEN WHEN:

DME NEEDED:

Notes:

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Alan Scott, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% - Medicare Beneficiary Identifier (MBI): 1U86AA7PN76 Effective Date: 1-1-2005

☐ Bill Medicaid 100% - Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____
Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____
Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____ Effective Date: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services.

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST
☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://texas.force.com/complaints/>, or by calling 1 800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714 9030 or faxed to 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complaint.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Alvin Scott
Beneficiary/Patient Signature

10/28/25
Date

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Relationship to Patient

Reason patient is unable to sign

[Signature]
Hospice Agency Representative Signature

10/28/25
Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Alvin Scott, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 10/28/25 (start of care date)

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5 00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentracio.com, phone number 888-315-0636, TTY number 711

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice)
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - ☐ Provided by the designated Hospice
 - ☐ Provided by another hospice under arrangements made by the designated Hospice
 - ☐ Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is _____

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo

Office Address: _____ NPI (if known) 1063664076

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☒ N/A Patient is non Medicare

☒ N/A I was transferred from _____ on _____

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice) Homage Hospice

I am electing the (1st, 2nd, 3rd, 4th, etc.) 1st benefit period with the effective date of election to begin on 10-28-25

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive

Mon Scott
Beneficiary/Patient Signature

10/28/25
Date

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign) _____

Date

Relationship to Patient _____

Reason patient is unable to sign _____

[Signature]
Hospice Agency Representative Signature

10/28/25
Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs
Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or
☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

Nina Scott
Signature of Beneficiary/Patient's Authorized Representative

10/28/25
Date

Representative's Relationship/Reason Patient Unable to Sign

Date

[Signature]
Signature of Witness/Hospice Representative

10/28/25
Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Alvin Scott.
I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

I. I understand the Hospice's obligations are as follows:

A. The Hospice staff will provide instructions to me as needed.

B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.

C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.

II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.

III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

Alvin Scott
Caregiver Signature

10/28/25
Date

[Signature]
Hospice Signature

10/28/25
Date

Patient Choice Statement

(To be completed for all new patients/guardians)

I, Alvin Scott (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice. I have selected _____ (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

Alvin Scott
Signature of Patient/Guardian

10/28/25
Date

[Signature]
Signature of Hospice Representative

10/28/25
Date

Hospice Wound Photography Consent

Alvin Scott

(Name of Patient)

_____ hereby consent and so authorize the taking of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: _____

Alvin Scott

Patient Signature (or other authorized signature)

10/28/25
Date

[Signature]

Witness Signature

10/28/25
Date

Relationship, if other than patient _____

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

Alvin Scott

Beneficiary/Patient/Authorized Representative Signature

10/28/25
Date

[Signature]

Hospice Agency Representative Signature

10/28/25
Date



—HOSPICE—

Homage Hospice
8204 Elnbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name *Alvin Scott*

Birth Date *5/0-1961*

Address

City

State

Zip

Home Phone

Business Ph

Cell

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

* Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List. * * *

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

Alvin Scott
Patient/Authorized Signature

Date

10/28/25

Medicare Secondary Payer Screening Form

Patient Name:

Scott, Alvin

SOC:

10282025

Date:

10282025

PART I

- Are you receiving Black Lung (BL) Benefits?
☒ No Go to 2.
☐ Yes BL is primary payer only for claims related to BL. Date benefits began: _____
- Are the services to be paid by a government research program?
☒ No Go to 3.
☐ Yes Government Research Program will pay primary benefits for these services.
- Has the Department of Veterans Affairs (DVA) authorized and agreed to pay for your care at this facility?
☒ No Go to 4.
☐ Yes DVA is primary for these services.
- Was the illness/injury due to a work-related accident/condition?
☒ No Go to Part II
☐ Yes Go to *Insurance Information Form* WC is primary payer only for related claims for work-related

PART II

- Was illness/injury due to a non-work-related accident?
☒ No Go to Part III
☐ Yes Go to *Insurance Information Form*

PART III

- Are you entitled to Medicare based on:

☒ Age- Go to PART IV

☐ Disability- Go to PART V

☐ End-Stage Renal Disease (ESRD)- Go to PART VI.

Complete ALL "PARTS" associated with the patient's selections.

PART IV - AGE

If the patient answers "no" to 2, Medicare is primary unless answered "yes" to questions in part I or II.

- Are you or your spouse currently employed?
Patient ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
Spouse ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
- Do you have group health plan (GHP) coverage based on your own or a spouse's current employer who employs 20 or > employees?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART V - DISABILITY

If the patient answers "no" to all, Medicare is primary unless answered "yes" to questions in part I or II.

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member's current employer who employs 100 or > employees?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART VI - ESRD

If the patient answers "no" to all in 1, Medicare is primary unless answered "yes" to questions in part I or II.

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
- Have you received a kidney transplant?
☒ No
☐ Yes Date of Transplant: _____
- Have you received maintenance dialysis treatments?
☒ No
☐ Yes Dialysis Start Date: _____ If app, Self-Dialysis Training Program Start Date: _____
- Are you within the 30-month coordination period that starts MM/DD/CCYY? (The 30-month coordination period starts the first day of the month an individual is eligible for Medicare (even if not yet enrolled in Medicare) because of kidney failure (usually the fourth month of dialysis). If the individual is participating in a self-dialysis training program or has a kidney transplant during the 3-month waiting period, the 30-month coordination period starts with the first day of the month of dialysis or kidney transplant.)
☒ No Medicare is Primary
☐ Yes
- Are you entitled to Medicare on the basis of either ESRD and age or ESRD and disability?
☒ No
☐ Yes
- Was your initial entitlement to Medicare (including simultaneous or dual entitlement) based on ESRD?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.
- Does the working aged or disability MSP provision apply (i.e., is the GHP already primary based on age or disability entitlement)?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.

Hospice Signature:

[Signature]

Date:

10282025



—HOSPICE—

REFERRAL/INTAKE FORM

Referral Date:	10-21-2025	Form Completed By:	Tina King
Referral Obtained by:	Carla Rolferson	Referral Source:	Carla Rolferson

PATIENT INFORMATION SUMMARY

Patient Name (Match Medicare Card):	Alvin Scott	Patient Address:	Glenn Heights Tx 75154
DOB:	5/10/1961	Facility & Room #:	1101 Mountain View St
SS#:	467-19-3962	County of Residence:	
Age:		Phone:	
Height/Weight:		Marital Status:	
Gender:		Religion:	
Race:		Allergies:	
Diagnosis:		Preferred Language:	
Name of Hospital (for Medical Records Request):			

COVERAGE & PLAN DETAILS

Medicare MBI:		Medicaid Effective Date:	
MBI Effective Date:		Medicare MBI:	
Other Insurance #:		Currently on Hospice Services?	
Insurance Phone/Address:		Currently on HH?	

CARE DIRECTIVES & SERVICE RECOGNITION

Funeral Home:		Veteran: Y/N	
Funeral Phone:		Military Branch:	
DNR: Y/N		Enrolled in VA: Y/N	
Attending MD:		Service Era:	
Attending Phone #:			
Continue as Attending: Y/N:			

DESIGNATED REPRESENTATIVES

Primary CG/POA Name:		Phone #:	
Relationship to Patient:		Alt. Phone #:	
Address:		Email Address:	
Emergency Contact Name:		Phone #:	
Relationship to Patient:		Alt. Phone #:	
Address:		Email Address:	

PT/FAMILY READY TO PROCEED WITH ADMISSION: ☐ YES ☐ NO IF NO, THEN WHEN:

DME NEEDED:

Notes:

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Alan Scott, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% - Medicare Beneficiary Identifier (MBI): 1U86AA7PN76 Effective Date: 1-1-2005

☐ Bill Medicaid 100% - Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____
Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____
Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____ Effective Date: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services.

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST
☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://texas.force.com/complaints/>, or by calling 1 800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714 9030 or faxed to 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complaint.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Alvin Scott
Beneficiary/Patient Signature

10/28/25
Date

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

Relationship to Patient

Reason patient is unable to sign

[Signature]
Hospice Agency Representative Signature

10/28/25
Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Alvin Scott, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 10/28/25 (start of care date)

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5 00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice)
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - ☐ Provided by the designated Hospice
 - ☐ Provided by another hospice under arrangements made by the designated Hospice
 - ☐ Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is _____

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo

Office Address: _____ NPI (if known) 1063664076

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☒ N/A Patient is non Medicare

☒ N/A I was transferred from _____ on _____

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice) Homeage Hospice

I am electing the (1st, 2nd, 3rd, 4th, etc.) 1st benefit period with the effective date of election to begin on 10-28-25

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive

Mon Scott
Beneficiary/Patient Signature

10/28/25
Date

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign) _____

Date

Relationship to Patient _____

Reason patient is unable to sign _____

[Signature]
Hospice Agency Representative Signature

10/28/25
Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs
Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or
☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

Nina Scott
Signature of Beneficiary/Patient's Authorized Representative

10/28/25
Date

Representative's Relationship/Reason Patient Unable to Sign

Date

[Signature]
Signature of Witness/Hospice Representative

10/28/25
Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Alvin Scott.
I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

I. I understand the Hospice's obligations are as follows:

A. The Hospice staff will provide instructions to me as needed.

B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.

C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.

II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.

III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

Alvin Scott
Caregiver Signature

10/28/25
Date

[Signature]
Hospice Signature

10/28/25
Date

Patient Choice Statement

(To be completed for all new patients/guardians)

I, Alvin Scott (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice. I have selected _____ (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

Alvin Scott
Signature of Patient/Guardian

10/28/25
Date

[Signature]
Signature of Hospice Representative

10/28/25
Date

Hospice Wound Photography Consent

Alvin Scott

(Name of Patient)

_____ hereby consent and so authorize the taking of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: _____

Alvin Scott

Patient Signature (or other authorized signature)

10/28/25
Date

[Signature]

Witness Signature

10/28/25
Date

Relationship, if other than patient _____

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

Alvin Scott

Beneficiary/Patient/Authorized Representative Signature

10/28/25
Date

[Signature]

Hospice Agency Representative Signature

10/28/25
Date



—HOSPICE—

Homage Hospice
8204 Elnbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name *Alvin Scott*

Birth Date *5/0-1961*

Address

City

State

Zip

Home Phone

Business Ph

Cell

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

* Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List. * * *

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

Alvin Scott
Patient/Authorized Signature

Date

10/28/25

Fax From Methodist Health Sys

Oct 22nd, 2025 2:45pm CST

From	(214) 947 - 8500
To	(469) 617 - 6998
Result	Fax Receive Successful
Sender Caller ID	Methodist Health Sys
Number of Pages	30
Transmission Time	19 minutes, 10 seconds



Fax Transmission

Methodist Health System

1441 N Beckley Ave.
Dallas, TX 75203
Phone: (214) 947-7600
Fax: (214) 947-7632

Date: 10/22/25
Time: 1:28 PM

Attn To:

To: Homage Hospice
Phone Number: 469-625-0705
Fax Number: 469-617-6998

From: Latoya N. Richmond
Phone:

Subject: Request for Medical Records

Comments: You are receiving this in response to a request for medical record information.

STATEMENT OF CONFIDENTIALITY: This electronic transmission contains information from Methodist Health System and should be considered confidential and privileged. The information contained in the above messages is intended only for the use of the individual(s) and entity(ies) above. If you are not the intended recipient, be aware that any disclosure, copying, distribution, or use of this information is prohibited. If you receive this transmission in error, please notify the Privacy Officer of Methodist Health System at (214) 947-4472 or by e-mail (PrivacyOfficer@mhd.com). Methodist Health System, its subsidiaries and affiliates hereby claim all applicable privileges related to the transmission of this communication.

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main**H&P Notes****H&P by McSpadden, Joshua R. at 7/11/2025 1317**

Author: McSpadden, Joshua R.

Service: Orthopedics

Author Type: Physician

Filed: 07/11/25 1318

Date of Service: 07/11/25 1317

Status: Signed

Editor: McSpadden, Joshua R. (Physician)

ORTHOPEDIC SURGERY CONSULT NOTE**Patient Name:** Alvin Dean Scott**Patient MRN:** MHD0399276**Date of Service:** 7/11/2025**Surgeon:** Joshua R. Mcspadden**Chief Complaint:** L arm pain**HISTORY OF PRESENT ILLNESS:**

Alvin Dean Scott is a 64 y.o. male who presents with L arm pain with hand numbness/tingling not relieved with conservative measures.

Review of Systems

As stated above. + msk pain L UE

Past Medical History**Past Medical History:**

Diagnosis	Date
• Abdominal aortic atherosclerosis	2017
• BMI 22.0-22.9, adult	04/08/2024
• BPH (benign prostatic hyperplasia)	05/22/2017
<i>CT abd/pel 2017 : Prostate is enlarged and contains calcifications.</i>	
• Chronic back pain	
• Chronic pain disorder	
• Dyslipidemia	
• ED (erectile dysfunction)	2005
• Fair tolerance for activity	
• Gastroesophageal reflux disease	
• GERD (gastroesophageal reflux disease)	2015
• History of echocardiogram	02/26/2022
<i>Normal chambers. Grade I diastolic dysfunction. Normal LV systolic function. EF 60. Normal valves.</i>	
• History of exercise stress test	02/13/2019
<i>Negative treadmill stress ECHO for inducible ischemia Reached 9 min on Bruce and 80% peak HR. Rest EF-60%. Stress EF-70%, Normal hemodynamic response to exercise. Good exercise capacity.</i>	
• History of nuclear stress test	03/02/2022
<i>Pharmacological stress test performed using 0.4 mg regadenoson over 10 second IV infusion. 1. Normal LV systolic function. LVEF estimated 59% with normal wall motion. 2. No scintigraphic evidence of reversible ischemia or prior infarct. 3. Visually no pathologic TID. 4. Overall low risk Lexiscan myocardial perfusion test.</i>	

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**H&P Notes (continued)**

- Hyperlipidemia 2015
- Hypertension 2015
- Low testosterone in male 2017
- Lumbar degenerative disc disease
MRI 8/18/2021 > Redemonstrated postoperative changes again noting enhancing granulation tissue on the LEFT at L3-L4 contributing to severe LEFT neuroforaminal stenosis. Persistent severe proximal RIGHT neuroforaminal stenosis on the basis of disk bulge and severe facet arthropathy. Overall, no significant interval change from 2019.
- Lumbar radiculopathy 2018
Failed back surgery syndrome (Chronic Stiff dull achy LBP since 1999 >> 2002- L4-S1 fusion - did not help with LBP Tried spinal cord stimulator ~ 2013 - eventually explanted >>> 2019 - lumbar spine surgery for "nerve compression")
- Neuropathy 2014
- Osteoarthritis of multiple joints 2016
- Seasonal allergic rhinitis
- Type 2 diabetes mellitus with diabetic chronic kidney disease (CMS/HCC) 2014
1/2/2024 : Hgb A1C 8.4
- Vision impairment 1973

Past Surgical History**Past Surgical History:**

Procedure	Laterality	Date
• ANKLE FRACTURE SURGERY forklift crush injury	Right	1986
• CARPAL TUNNEL RELEASE	Right	2005
• COLONOSCOPY by Dr William Major > diverticulosis		03/29/2016
• FINGER MASS EXCISION ganglion cyst ? 2nd digit DIP joint	Right	2016
• INGUINAL HERNIA REPAIR Open @ Parkland	Right	1993
• INGUINAL HERNIA REPAIR Open Recurrent Right Inguinal Hernia Repair with kugel mesh by William Major, MD @ Baylor Waxahachie surgery center	Right	04/05/2016
• LAMINECTOMY LUMBAR SPLINE W/ PLACEMENT SPINAL CORD STIMULATOR SCS implantation and eventual explantation date?		2013
• LUMBAR EPIDURAL INJECTION and SI injections BUMC		07/2021
• LUMBAR FUSION L4-5, L5-S1 by Dr Paul Vaughn		2005
• LUMBAR LAMINECTOMY L3-4 laminotomy, partial facetectomy and far right lateral discectomy for radicular pain		02/20/2019
• LUMBAR SPINE HARDWARE REMOVAL by Dr Dr. Paul Vaughn		2006
• ROTATOR CUFF REPAIR	Right	2009

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**H&P Notes (continued)**

- | | | |
|---|-------|------|
| • ROTATOR CUFF REPAIR
by Dr John Wey, MD | Left | 2022 |
| • ROTATOR CUFF REPAIR
By: Dr. John Wey, MD | Right | 2021 |

Family History**Family History**

Problem	Relation	Age of Onset
• Cancer	Mother	
• Lung cancer	Mother	
• Kidney cancer	Mother	
• Diabetes	Mother	
• Hypertension	Mother	
• Drug abuse	Father	
• Cancer	Sister	
• Heart disease	Brother	
• Gout	Brother	

Social History

- Tob/etoh/drugs: none

Medications

@CMEDOP@

Scheduled Meds:ceFAZolin, 2 g, slow IV push, Once**Continuous Infusions:**sodium chloride 0.9 %, 50 mL/hr, Last Rate: 50 mL/hr (07/11/25 1149)**PRN Meds:****Allergies****Allergies**

Allergen	Reactions
• Vancomycin	Hives

Physical Exam**Most Recent Vital Signs:** BP (!) 153/91 | Pulse 68 | Temp 97.6 °F (36.4 °C) (Temporal) | Resp 18 | Ht 5'

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**H&P Notes (continued)**8" (1.727 m) | Wt 150 lb 4.8 oz (68.2 kg) | SpO2 97% | BMI 22.85 kg/m²**Gen:** NAD, AAOx3**Chest:** symmetrical rise/fall with respirations**MSK: LUE:** n/v intact except decr sens med and uln N distribution**Imaging:****EMG shows L mod cub and carp tun****Labs:**

@LABRCNT(WBC:2,HGB:2,HCT:2,PLT:2)@

@LABRCNT(NA,K,CL,CO2,BUN,CREA,GLU,CA,MG,PHOS)@

@LABRCNT(PT,INR,PTT)@

@LABRCNT(ESR)@

@LABRCNT(CRP)@

Micro:

No components found for: "UAWBC", "UARBC", "LEU", "NIT"

ASSESSMENT/PLAN:

L cub tun, L carp tun

OR for L cub tun rel and carp tun rel

This procedure has been fully reviewed with the patient and written informed consent has been obtained.

Joshua R. Mcspadden**Orthopedic Surgery****7/11/2025**

Electronically signed by McSpadden, Joshua R. at 07/11/25 1318

Clinical Notes**Op Note****McSpadden, Joshua R. at 7/11/2025 1432**

Author: McSpadden, Joshua R.

Service: Orthopedics

Author Type: Physician

Filed: 07/11/25 1829

Date of Service: 07/11/25 1432

Status: Signed

Editor: McSpadden, Joshua R. (Physician)

DATE OF PROCEDURE: 07/11/2025

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Clinical Notes (continued)**

PREOPERATIVE DIAGNOSIS: Left cubital tunnel syndrome, left carpal tunnel syndrome.

POSTOPERATIVE DIAGNOSIS: Left cubital tunnel syndrome, left carpal tunnel syndrome.

PROCEDURE PERFORMED: Left cubital tunnel release and left carpal tunnel release.

SURGEON: Joshua R. McSpadden, DO

ASSISTANT: None.

ANESTHESIA: General with local 0.5% Marcaine plain.

COMPLICATIONS: None.

ESTIMATED BLOOD LOSS: 10 mL

SPECIMENS: None.

IMPLANTS: None.

FINDINGS: Ulnar nerve entrapment at the cubital tunnel and median nerve entrapment with thickened transverse carpal ligament of the carpal tunnel.

INDICATIONS: Above diagnosis with significant pain interfering with activities of daily living, quality of life, not relieved with conservative measures. EMG confirming moderate carpal and cubital tunnel.

Preoperatively, the patient was consented after discussing the risks, benefits and alternatives including but not limited to infection, bleeding, blood clots, nerve and artery damage, loss of limb, loss of life. The patient demonstrated understanding and wished to proceed. The patient was transferred to the operating room, was placed supine on the operating room table. All bony prominences well padded. Timeout was done, confirming correct patient, correct extremity, correct procedure. IV antibiotics given prior to the incision.

Left arm was outstretched on a hand table. Nonsterile tourniquet was placed on the left upper arm. The patient was sterilely prepped and draped. Esmarch used to exsanguinate the extremity and tourniquet inflated to 250 mmHg, which was up for the entirety of the procedure for 30 minutes.

I addressed the cubital tunnel first, with an approximately 4 cm curvilinear incision in between the medial epicondyle and the olecranon, went through skin, subcutaneous tissue, down to the intermuscular septum proximally and identified the ulnar nerve. I released it from proximal to distal through the cubital tunnel itself, where it was significantly constricted in this fascia, but also the FCU fascia distally had significant amount of constriction as well. I fully released the nerve and then took the elbow through gentle passive range of motion with no significant snapping of the nerve over the medial epicondyle. I maintained hemostasis with bipolar electrocautery. The nerve was intact and in good continuity and I thoroughly irrigated with sterile saline and closed the skin with 2-0 Vicryl, 3-0 Monocryl and Dermabond. Injected 20 mL 0.5% Marcaine plain.

I then turned my attention to the carpal tunnel, where I made an approximately 2 cm incision over the carpal tunnel in line with the radial border of the ring finger, not extending past Kaplan's cardinal line distally, went through skin, subcutaneous tissue down to the palmar fascia, swept the palmaris brevis muscle radially and identified the transverse carpal ligament, which was sharply incised and then extended both distally and proximally, while protecting neurovascular structures. I fully released the nerve and it was in good continuity and the transverse carpal ligament was significantly thickened.

I used bipolar electrocautery for hemostasis, thoroughly irrigated with sterile saline, closed the skin with 3-0 nylon, placed a sterile dressing on the hand as well as the elbow, as well as compressive Ace wrap. The patient was extubated and transferred to PACU in stable condition with no complications.

DICTATED BY: JOSHUA R. MCSPADDEN DO

D: 07/11/2025 14:32:07

Printed on 10/22/25 1:28 PM

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Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Clinical Notes (continued)**

T: 07/11/2025 15:51:00

Epic Job #: 332420493

Dictation Job #: 19270327

JOSHUA R. MCSADDEN DO

Electronically signed by McSpadden, Joshua R. at 07/11/25 1829

Labs**CBC w/ Auto Differential (Completed)**Electronically signed by: **Simpson, James, RN on 07/02/25 1715**Status: **Completed**

Ordering user: Simpson, James, RN 07/02/25 1715

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine 07/02/25 -

Class: Lab Collect

Quantity: 1

Specimen Information

ID	Type	Source	Collected By
—	Blood	Blood, Venous	—

Basic Metabolic Profile (Completed)Electronically signed by: **Simpson, James, RN on 07/02/25 1715**Status: **Completed**

Ordering user: Simpson, James, RN 07/02/25 1715

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine 07/02/25 -

Class: Lab Collect

Quantity: 1

Specimen Information

ID	Type	Source	Collected By
—	Blood	Blood, Venous	—

Basic Metabolic Profile (Final result)Electronically signed by: **Simpson, James, RN on 07/02/25 1715**Status: **Completed**

Ordering user: Simpson, James, RN 07/02/25 1715

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine Once 07/02/25 1716 - 1 occurrence

Class: Unit Collect

Quantity: 1

Lab status: Final result

Instance released by: Simpson, James, RN 7/2/2025 5:16 PM

Specimen Information

ID	Type	Draw Type	Source	Collected By
25LL-184C00103	Blood	Venipuncture	Blood, Venous	Kimball-East, Lauren 07/03/25 1409

Basic Metabolic Profile (Abnormal)

Resulted: 07/03/25 1423, Result status: Final result

Ordering provider: McSpadden, Joshua R. 07/02/25 1716

Order status: Completed

Filed by: Lab, Background User 07/03/25 1423

Collected by: Kimball-East, Lauren 07/03/25 1409

Resulting lab: MLMC LAB

CLIA number: 45D2199140

Components

Component	Value	Reference Range	Flag	Lab
Sodium	138	135 - 148 mmol/L	—	LL
Potassium	4.0	3.4 - 5.1 mmol/L	—	LL
Chloride	102	95 - 106 mmol/L	—	LL

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Labs (continued)**

CO2	28	22 - 31 mmol/L	—	LL
BUN	18	10 - 25 mg/dL	—	LL
Creatinine	0.80	0.70 - 1.40 mg/dL	—	LL
Glucose	115	70 - 110 mg/dL	H ^	LL
Calcium	9.2	8.4 - 10.2 mg/dL	—	LL
Anion Gap	8	8 - 16 meq/L	—	LL
eGFR	>60.0	>60.0 mL/min/1.73 m2	—	LL

Comment:

* Estimated Glomerular Filtration Rate (eGFR) comment:

Calculation: CKD-EPI Creatinine Equation (2021)

An estimated GFR is not accurate in children, pregnant women, people at the extremes of body-type (e.g. morbidly obese or malnourished), near the normal range (>60), or when the creatinine is not stable."

BUN/Creatinine Ratio	22.50	12.00 - 20.00	H ^	LL
Osmolality Calc	269	261 - 280	—	LL
		MOSM/kg		
Albumin	4.4	3.5 - 5.7 g/dL	—	LL
Corrected Calcium	8.9	8.4 - 10.2 mg/dL	—	LL

Comment:

Ionized calcium measurement is recommended for the following patient populations as formulaic corrected calcium may not accurately estimate ionized calcium: Critically ill surgical patients, renal failure and hemodialysis patients, primary hyperparathyroidism patients, patients having or suspected of having multiple myeloma and the very elderly.

Hemolysis	<15.0	<151	—	LL
Icterus	<2.0	<7	—	LL
Turbidity	<20.0	<21	—	LL

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
37 - LL	MLMC LAB	Unknown	1201 E US Hwy 287 MIDLOTHIAN TX 76065	06/12/20 1003 - Present

CBC w/ Auto Differential (Final result)Electronically signed by: **Simpson, James, RN on 07/02/25 1715**Status: **Completed**

Ordering user: Simpson, James, RN 07/02/25 1715

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine Once 07/02/25 1716 - 1 occurrence

Class: Unit Collect

Quantity: 1

Lab status: Final result

Instance released by: Simpson, James, RN 7/2/2025 5:16 PM

Specimen Information

ID	Type	Draw Type	Source	Collected By
25LL-184H00063	Blood	Venipuncture	Blood, Venous	Kimball-East, Lauren 07/03/25 1409

CBC w/ Auto Differential (Abnormal)

Resulted: 07/03/25 1412, Result status: Final result

Ordering provider: McSpadden, Joshua R. 07/02/25 1716

Order status: Completed

Filed by: Lab, Background User 07/03/25 1412

Collected by: Kimball-East, Lauren 07/03/25 1409

Resulting lab: MLMC LAB

CLIA number: 45D2199140

Components

Component	Value	Reference Range	Flag	Lab
WBC	5.9	3.8 - 10.6 X10 ³ /uL	—	LL
RBC	4.38	4.50 - 5.90 X10 ⁶ /uL	L *	LL
Hemoglobin	14.8	13.5 - 17.5 g/dL	—	LL

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Hematocrit	40.9	40.2 - 52.0 %	—	LL
MCV	93.4	80.0 - 100.0 FL	—	LL
MCH	33.8	26.0 - 34.0 pg	—	LL
MCHC	36.2	31.1 - 36.6 g/dL	—	LL
RDW-CV	12.5	11.5 - 14.5 %	—	LL
Platelet Count	176	130 - 400	—	LL
		X10 ³ /uL		
MPV	9.6	9.0 - 13.0 FL	—	LL
Neutrophils %	67.2	36.0 - 66.0 %	H [^]	LL
Lymphocytes %	22.1	24.0 - 44.0 %	L ^w	LL
Monocytes %	6.6	0.1 - 10.0 %	—	LL
Eosinophils %	3.5	<=5.0 %	—	LL
Basophils %	0.3	<=2.0 %	—	LL
Neutrophils # Count	4.0	1.4 - 7.3 10 ³ /uL	—	LL
Lymphocytes # Count	1.3	0.9 - 4.8 10 ³ /uL	—	LL
Monocytes Absolute Count	0.4	0.1 - 1.1 10 ³ /uL	—	LL
Eosinophils Absolute Count	0.2	<=0.6 10 ³ /uL	—	LL
Basophils # Count	0.0	0.0 - 0.2 10 ³ /uL	—	LL
Immature Granulocyte %	0.3	0.0 - 0.5 %	—	LL
Immature Gran #	0.02	0.00 - 0.50	—	LL
		10 ³ /uL		
nRBC %	0.0	<=1.0 /100WBC	—	LL
nRBC #	0.0	0 10 ³ /uL	—	LL
Absolute Neutrophils/Lymphocytes Ratio	3.08	<=3.50	—	LL

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
37 - LL	MLMC LAB	Unknown	1201 E US Hwy 287 MIDLOTHIAN TX 76065	06/12/20 1003 - Present

POCT Glucose (Final result)Electronically signed by: Interface, Results From Lab Instruments - To Beaker, Poct In
on 07/11/25 1147

Status: Completed

Ordering user: Interface, Results From Lab Instruments - To
Beaker, Poct In 07/11/25 1147

Authorized by: Unknown, Doctor

Ordering mode: Standard

Frequency: Routine Once 07/11/25 1150 - 1 occurrence

Class: Point Of Care

Quantity: 1

Lab status: Final result

Instance released by: Interface, Results From Lab Instruments - To Beaker, Poct In (auto-released) 7/11/2025 11:49 AM

Specimen Information

ID	Type	Source	Collected By
25LL-192P00046	Blood	Blood, Capillary	07/11/25 1147

POCT Glucose (Abnormal)

Resulted: 07/11/25 1149, Result status: Final result

Order status: Completed

Filed by: Lab, Background User 07/11/25 1149

Collected by: 07/11/25 1147

Resulting lab: MLMC LAB

CLIA number: 45D2199140

Components

Component	Value	Reference Range	Flag	Lab
Glucose Blood, POC	169	70 - 110 mg/dL	H [^]	LL

Testing Performed By

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Labs (continued)**

Lab - Abbreviation	Name	Director	Address	Valid Date Range
37 - LL	MLMC LAB	Unknown	1201 E US Hwy 287 MIDLOTHIAN TX 76065	06/12/20 1003 - Present

Other Orders**Medications****sodium chloride 0.9 % (NS) infusion (Discontinued)**Electronically signed by: **Luque, Heather, RN on 07/08/25 1201**Status: **Discontinued**

Ordering user: Luque, Heather, RN 07/08/25 1201

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine Continuous 07/11/25 1145 - 20 hours

Class: Normal

Released by: Olivares, Lizet N., RN 07/11/25 1125

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Olivares, Lizet N., RN 07/11/25 1125 for Placing Order

Questionnaire

Question	Answer
To optimize patient care and safety, a 1-LITER DURATION will be applied by pharmacy where appropriate. Is this sufficient for this order?	Yes

Package: 0338-0049-04

Status

Kolawole, Foluke, PharmD 07/11/25 1136 (End: Until discontinued to 07/12/25 0744)

Olivares, Lizet N., RN 07/11/25 1149 (End: 07/12/25 0744 to 07/11/25 1911)

acetaminophen (TYLENOL) tablet 1,000 mg (Completed)Electronically signed by: **Luque, Heather, RN on 07/08/25 1202**Status: **Completed**

Ordering user: Luque, Heather, RN 07/08/25 1202

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Per protocol

Frequency: Routine Once 07/11/25 1130 - 1 occurrence

Class: Normal

Released by: Olivares, Lizet N., RN 07/11/25 1125

Acknowledged: Olivares, Lizet N., RN 07/11/25 1125 for Placing Order

Admin instructions: COMMON SIDE EFFECTS: Nausea/Vomiting, Diarrhea or constipation, Weakness and Loss of appetite

Package: 0904-6730-61

ceFAZolin (ANCEF) 2 g in sterile water 20 mL injection (Completed)Electronically signed by: **Luque, Heather, RN on 07/08/25 1203**Status: **Completed**

Ordering user: Luque, Heather, RN 07/08/25 1203

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Transcribed from paper

Frequency: Routine Once 07/11/25 1130 - 1 occurrence

Class: Normal

Indications of use: prevention of perioperative infection

Released by: Olivares, Lizet N., RN 07/11/25 1125

Acknowledged: Olivares, Lizet N., RN 07/11/25 1125 for Placing Order

Mixture Ingredients

Medication	Ordered Dose	Calculated Dose
ceFAZolin (ANCEF)	2 g	2 g
sterile water	20 mL	20 mL

Admin instructions: For slow IV push, reconstitute each gram with 10 mL SWFI and give over 2-5 minutes.

Package: 25021-101-10, 0409-4887-17

sodium chloride 0.9 % (NS) irrigation solution - ADS Override Pull (Completed)Electronically signed by: **Shingles, Candace, RN on 07/11/25 1300**Status: **Completed**

Ordering user: Shingles, Candace, RN 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Other Orders (continued)**

Package: 0338-0048-04

bupivacaine PF (MARCAINE) 0.25 % (2.5 mg/mL) injection - ADS Override Pull (Discontinued)Electronically signed by: **Shingles, Candace, RN on 07/11/25 1300**Status: **Discontinued**

Ordering user: Shingles, Candace, RN 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Discontinued by: Interface, Ads Dispense 07/11/25 1301

[Returned to ADS]

Admin instructions: Created by cabinet override

COMMON SIDE EFFECTS: Injection site irritation, Sudden vision change,

Imbalance and Weakness

Medication comments: Created by cabinet override

Package: 0143-9333-01

ondansetron (ZOFTRAN) 4 mg/2 mL injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

Package: 0409-4755-18

propofol (DIPRIVAN) 10 mg/mL injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

Package: 25021-608-20

dexAMETHasone (DECADRON) 4 mg/mL injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

Package: 25021-052-01

lidocaine PF (cardiac) (XYLOCAINE) 100 mg/5 mL (2 %) injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

Package: 76329-3390-1

fentaNYL PF (SUBLIMAZE) 50 mcg/mL injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

COMMON SIDE EFFECTS: Shortness of breath, Drowsiness, Nausea/Vomiting, and Constipation

Medication comments: Created by cabinet override

Package: 0641-6027-01

midazolam (VERSED) 1 mg/mL injection - ADS Override Pull (Completed)Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1300**Status: **Completed**

Ordering user: Yang, Diana Peggy, MD 07/11/25 1300

Ordering mode: Standard

Frequency: 07/11/25 1300 - 1 occurrence

Admin instructions: Created by cabinet override

COMMON SIDE EFFECTS: Dizziness/Drowsiness, Confusion and Dry mouth

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07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Other Orders (continued)**

Medication comments: Created by cabinet override
 Package: 0641-6209-01

bupivacaine PF (MARCAINE) 0.5 % (5 mg/mL) injection - ADS Override Pull (Completed)

Electronically signed by: **Shingles, Candace, RN on 07/11/25 1301** Status: **Completed**
 Ordering user: Shingles, Candace, RN 07/11/25 1301 Ordering mode: Standard
 Frequency: 07/11/25 1301 - 1 occurrence
 Admin instructions: Created by cabinet override
 COMMON SIDE EFFECTS: Injection site irritation, Sudden vision change,
 Imbalance and Weakness
 Medication comments: Created by cabinet override
 Package: 63323-466-39

lidocaine 1 % (XYLOCAINE) 10 mg/mL (1 %) Injection - ADS Override Pull (Discontinued)

Electronically signed by: **Shingles, Candace, RN on 07/11/25 1301** Status: **Discontinued**
 Ordering user: Shingles, Candace, RN 07/11/25 1301 Ordering mode: Standard
 Frequency: 07/11/25 1301 - 1 occurrence Discontinued by: Interface, Ads Dispense 07/11/25 1415
 [Returned to ADS]
 Admin instructions: Created by cabinet override
 Medication comments: Created by cabinet override
 Package: 0409-4276-01

bupivacaine PF (MARCAINE) 0.5 % (5 mg/mL) Injection (Discontinued)

Electronically signed by: **McSpadden, Joshua R. on 07/11/25 1434** Status: **Discontinued**
 Mode: Ordering in Verbal readback mode Communicated by: Shingles, Candace, RN
 Ordering user: Shingles, Candace, RN 07/11/25 1357 Ordering provider: McSpadden, Joshua R.
 Authorized by: McSpadden, Joshua R. Ordering mode: Verbal readback
 Frequency: Routine PRN 07/11/25 1356 - 07/11/25 1422 Class: Normal
 Discontinued by: Yang, Diana Peggy, MD 07/11/25 1422 [Patient Discharge]
 Acknowledged: Shingles, Candace, RN 07/11/25 1357 for Placing Order
 Package: 63323-466-39

sodium chloride 0.9 % (NS) irrigation solution (Discontinued)

Electronically signed by: **McSpadden, Joshua R. on 07/11/25 1434** Status: **Discontinued**
 Mode: Ordering in Verbal readback mode Communicated by: Shingles, Candace, RN
 Ordering user: Shingles, Candace, RN 07/11/25 1357 Ordering provider: McSpadden, Joshua R.
 Authorized by: McSpadden, Joshua R. Ordering mode: Verbal readback
 Frequency: Routine PRN 07/11/25 1357 - 07/11/25 1422 Class: Normal
 Discontinued by: Yang, Diana Peggy, MD 07/11/25 1422 [Patient Discharge]
 Acknowledged: Shingles, Candace, RN 07/11/25 1357 for Placing Order
 Package: 0338-0048-03

HYDROMORPHONE (DILAUDID) 0.5 mg/0.5 mL Injection 0.2 mg (Discontinued)

Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1336** Status: **Discontinued**
 Ordering user: Yang, Diana Peggy, MD 07/11/25 1336 Ordering provider: Yang, Diana Peggy, MD
 Authorized by: Yang, Diana Peggy, MD Ordering mode: Standard
 PRN reasons: other
 PRN Comment: mild or moderate pain (1-6/10)
 Frequency: STAT q8 min PRN 07/11/25 1417 - 07/11/25 1911 Class: Normal
 Released by: Velasquez, Jazmine A., RN 07/11/25 1417 Discontinued by: Discharge Provider, Automatic 07/11/25 1911
 [Patient Discharge]
 Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order
 Admin instructions: Maximum total dose is 2 mg while in PACU. If morphine ordered and
 ineffective, hydromorphone is 2nd line.
 Package: 76045-009-96

HYDROMORPHONE (DILAUDID) 0.5 mg/0.5 mL Injection 0.3 mg (Discontinued)

Electronically signed by: **Yang, Diana Peggy, MD on 07/11/25 1336** Status: **Discontinued**
 Ordering user: Yang, Diana Peggy, MD 07/11/25 1336 Ordering provider: Yang, Diana Peggy, MD
 Authorized by: Yang, Diana Peggy, MD Ordering mode: Standard
 PRN reasons: sev pain (7-10/10)

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Other Orders (continued)**

Frequency: STAT q8 min PRN 07/11/25 1417 - 07/11/25 1911

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Admin instructions: Maximum total dose is 2 mg while in PACU. If morphine ordered and ineffective, hydromorphone is 2nd line.

Package: 76045-009-96

fentanyl PF (SUBLIMAZE) injection 25 mcg (Discontinued)

Electronically signed by: Yang, Diana Peggy, MD on 07/11/25 1336

Status: Discontinued

Ordering user: Yang, Diana Peggy, MD 07/11/25 1336

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Standard

PRN reasons: sev pain (7-10/10)

Frequency: STAT q5 min PRN 07/11/25 1417 - 07/11/25 1911

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Admin instructions: COMMON SIDE EFFECTS: Shortness of breath, Drowsiness, Nausea/Vomiting, and Constipation

Package: 0641-6027-01

HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet 1 tablet (Discontinued)

Electronically signed by: Yang, Diana Peggy, MD on 07/11/25 1336

Status: Discontinued

Ordering user: Yang, Diana Peggy, MD 07/11/25 1336

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Standard

PRN reasons: mod pain (4-6/10)

Frequency: Routine q4h PRN 07/11/25 1417 - 07/11/25 1911

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Package: 60687-396-11

ondansetron (ZOFTRAN) injection 4 mg (Discontinued)

Electronically signed by: Yang, Diana Peggy, MD on 07/11/25 1336

Status: Discontinued

Ordering user: Yang, Diana Peggy, MD 07/11/25 1336

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Standard

PRN reasons: nausea

Frequency: STAT q30 min PRN 07/11/25 1417 - 2 occurrences

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Admin instructions: Ondansetrol injection is considered option #1-if ineffective, progress to the next selected option for nausea.

Package: 0409-4755-18

meperidine PF (DEMEROL) injection 6.25 mg (Discontinued)

Electronically signed by: Yang, Diana Peggy, MD on 07/11/25 1336

Status: Discontinued

Ordering user: Yang, Diana Peggy, MD 07/11/25 1336

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Standard

PRN reasons: shivering

Frequency: STAT q10 min PRN 07/11/25 1417 - 4 occurrences

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Package: 0641-6052-01

hydRALAZINE (APRESOLINE) injection 5 mg (Discontinued)

Electronically signed by: Yang, Diana Peggy, MD on 07/11/25 1336

Status: Discontinued

Ordering user: Yang, Diana Peggy, MD 07/11/25 1336

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Standard

PRN Comment: SBP > 160 or DBP > 90 (first choice)

Frequency: Routine q10 min PRN 07/11/25 1417 - 5

Class: Normal

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 7/11/2025, D/C: 7/11/2025

07/11/2025 - Admission (Discharged) in Methodist Midlothian Medical Center OR Main (continued)**Other Orders (continued)**

occurrences

Released by: Velasquez, Jazmine A., RN 07/11/25 1417

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1417 for Placing Order

Admin instructions: COMMON SIDE EFFECTS: Dizziness, Headache, Nausea, and Weight gain

Package: 63323-614-01

HYDROcodone-acetaminophen (NORCO) 5-325 mg per tablet (Active)Electronically signed by: **McSpadden, Joshua R. on 07/11/25 1427**Status: **Active**

Ordering user: McSpadden, Joshua R. 07/11/25 1427

Ordering provider: McSpadden, Joshua R.

Authorized by: McSpadden, Joshua R.

Ordering mode: Standard

PRN reasons: mild pain moderate pain

Frequency: q8h PRN 07/11/25 - Until Discontinued

Class: Normal

Diagnoses

Left carpal tunnel syndrome [G56.02]

Cubital tunnel syndrome on left [G56.22]

Medication comments:

Modified from: HYDROCODONE-ACETAMINOPHEN ORAL

oxyCODONE (ROXICODONE) immediate release tablet 5 mg (Discontinued)Electronically signed by: **Yang, Diana Peggy, MD on 07/12/25 1848**Status: **Discontinued**

Mode: Ordering in Verbal readback mode

Communicated by: Velasquez, Jazmine A., RN

Ordering user: Velasquez, Jazmine A., RN 07/11/25 1526

Ordering provider: Yang, Diana Peggy, MD

Authorized by: Yang, Diana Peggy, MD

Ordering mode: Verbal readback

PRN reasons: mod pain (4-6/10) sev pain (7-10/10)

Frequency: Routine q4h PRN 07/11/25 1526 - 07/11/25 1911

Class: Normal

Released by: Velasquez, Jazmine A., RN 07/11/25 1526

Discontinued by: Discharge Provider, Automatic 07/11/25 1911
[Patient Discharge]

Acknowledged: Velasquez, Jazmine A., RN 07/11/25 1526 for Placing Order

Package: 0406-0552-23

oxyCODONE (ROXICODONE) 5 mg immediate release tablet - ADS Override Pull (Completed)Electronically signed by: **Velasquez, Jazmine A., RN on 07/11/25 1527**Status: **Completed**

Ordering user: Velasquez, Jazmine A., RN 07/11/25 1527

Ordering mode: Standard

Frequency: 07/11/25 1527 - 1 occurrence

Admin instructions: Created by cabinet override

Medication comments: Created by cabinet override

Package: 0406-0552-23

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main**H&P Notes****Interval H&P Note by Wey, John T., MD at 8/30/2018 0936**

Author: Wey, John T., MD

Service: Orthopedics

Author Type: Physician

Filed: 08/30/18 0936

Date of Service: 08/30/18 0936

Status: Signed

Editor: Wey, John T., MD (Physician)

H&P reviewed. The patient was examined and there are no changes to the H&P.

Electronically signed by Wey, John T., MD at 08/30/18 0936

Source Note: H&P

Author: Wey, John T., MD

Service: —

Author Type: Physician

Filed: 08/29/18 1347

Date of Service: 08/29/18 1347

Status: Signed

Editor: Patton, Kasey G., RN (Registered Nurse)

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)

H&P Notes (continued)

H&P - Scan - Scan on 8/29/2018 1347 (below)

Patient: Beverly, Beverly To: OPS (1574894062) 8/29/2018
 Patient: SCOTT, ALVIN (M) 8/29/2018
 DOB: 05/10/1961 (M)
 14:09 8/29/18 ET Pg 3-3
History and physical
 08/29/2018

L. and W. Orthopaedics
 Patient: SCOTT, ALVIN (M)
 1121 MOUNTAIN VIEW CT
 RED OAK, TX 75154
 (469) 760-8382
 ALSCOTT1214@YAHOO.COM
 DOB: 05/10/1961 (M)
 Previous First Name:
 Previous Last Name:
 Race: Patient Declined
 Language: English
 Ethnicity: Patient Declined
 Sexual Orientation:
 Gender:
 Identity:
 Encounter ID: 082918-47868763
 Primary Inst: United Methodist
 Location: L. and W. Orthopaedics
 2801 E. PUGH ST. SUITE 201
 Richardson, TX 75081-1551
 (972) 468-4791 Ext 0
 Provider: JAMES T. WRIGHT MD
 Referring:

Subjective

Chief Complaint: bilateral shoulder pain

History of Present Illness - bilateral shoulder pain

Location:
 Reported: bilateral shoulders.

Quality:
 Reported: moderate to severe.

Severity:
 Reported: 8/10 pain.

Duration:
 Reported: about 2-3 weeks.

Context:
 Reported: my husband and I both have shoulder pain, but he has more severe pain than I do.

Modifying Factors:
 Reported: no change.

Associated Signs and Symptoms:
 Reported: no change.

Scott, Alvin Dean
 MRN: MHD0399276, Adm: 08/14/2018
 5/10/1961, 57 y.o., M, Civil: 123512915

Medication History:

Date	Medication	Sig	#	Refill	Status
07/08/2018	metformin 1,000 mg tablet	1 tablet by mouth daily		0	Active

Social History:

Substance	Usage Status	Last Used	Daily Usage	Packaging	Years of Use	History of Attempts	Last Cessation #/s
Tobacco	Never smoked			Packs			
Alcohol	Usage Status	Last Used	Usage Number	Packaging	Measurement Time		
Beer	Social						

Page 16 of 30
 Patient: SCOTT, ALVIN (M) 8/29/2018
 DOB: 05/10/1961 (M)
 14:09 8/29/18 ET Pg 3-3

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**H&P Notes (continued)**

PATIENT: Beverly, Scott, Alvin (13724084862) 14:18 28/28/18 E1 Pg 4-5

Medical History:

Positive History:

Condition	Physician	Hospitalization/Op Date	Hospital
Diabetes			

Surgical History:

Positive History:

Condition	Physician	Hospitalization/Op Date	Hospital
Repair of rotator cuff by suture (procedure)			

Family History:

Type	Sex	Status	Age	Name	Negative History	Positive History	Direct Date	Positive Comment	Negative Comment
Father	M	Alive				Diabetes			
Mother	F	Alive				Hypertension			
						Diabetes			
						Hypertension			

Review of Systems:

Objective

Vital Signs:

Physical Exam:

Constitutional

The patient is awake, alert, well developed, well groomed and well nourished.

Cardiovascular

The heart rate is normal, the rhythm is regular. There is no palpable edema of the lower extremities. The peripheral artery pulses are 2+.

Lymphatics

Lymphadenitis:

There is no upper or lower extremity lymphadenitis.

Musculoskeletal

Left Shoulder Upper

Inspection/Palpation

General:

Upon inspection there are no deformities or misalignment of bones. There are no ecchymosis, erythema, scabbing, subcutaneous nodules, or signs of muscle atrophy. Upon palpation there is no swelling, nodules, tenderness, or crepitus. The bony landmarks are normal and there is physical continuity of the anatomic structures.

Alignment

The patient's structural posture is symmetrical. The bony landmarks of the two shoulders are aligned.

Range of Motion

Abduction:

Active range of motion of the shoulder is abducted at 90 degrees.

Flexion:

Active range of motion of the shoulder is flexed at 120 degrees.

Scott, Alvin Dean
MRN: MHD0399276/DOB: 1060147321
5/10/1961 57 y.o. M / SN: 122812915

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MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) In Mrmc Or Main (continued)

H&P Notes (continued)

Fr:Durale, Beverly To:OPS (18774964902) SCOTT ALVIN (6619116A1) 14:11 06/29/16 ET Pg 5-9

06/29/16

Internal Rotation:
Active range of motion of the shoulder is internal rotation is 90 degrees.

External Rotation:
Active range of motion of the shoulder is external rotation is 90 degrees.

Muscle Testing

Strength testing of the major motor muscles of the left shoulder and upper arm is graded at 5/5

Special Tests

Special Tests Rotator Cuff Disorders:
The Neer impingement sign is positive for rotator cuff tendinitis.

Special Tests Biceps Tendon:
The Speed test is negative.

Right Shoulder Upper

Inspection Palpation:

General:
No gross deformities are present; no obvious malalignments or lesions. There are no observable, palpable abnormalities or bony irregularities. A full range of motion is observed upon inspection. No swelling, effusion, warmth, redness, or crepitus. The sensory examination was normal and there is no significant tenderness of the osseous structures. Tenderness associated

Alignment:
The patient's shoulder girdle is symmetrical. The bony landmarks of the two shoulders are aligned.

Range of Motion:

Abduction:
Active range of motion of the shoulder in abduction is 90 degrees.

Flexion:
Active range of motion of the shoulder in flexion is approximately 90 degrees.

Internal Rotation:
Active range of motion of the shoulder in internal rotation is approximately 90 degrees.

External Rotation:
Active range of motion of the shoulder in external rotation is approximately 90 degrees.

Muscle Testing

Strength testing of the major motor muscles of the right shoulder and upper arm is graded at 5/5

Special Tests

Special Tests Rotator Cuff Disorders:
The Neer impingement sign is positive for rotator cuff tendinitis.

Special Tests Biceps Tendon:
The Speed test is positive. There is pain and difficulty moving the right forearm.

Left Elbow Forearm:

Special Tests

Nerve Compression Syndromes:
The elbow flexor test is negative; there is no numbness/tingling of nerves distal to the elbow flexor and wrist flexor.

Neurologic

The deep tendon reflexes of the upper extremities are symmetrically brisk and graded at 2+. Sensory testing for light touch is decreased anterior aspect left little finger. Preserved distal left ulnar sensation.

Assessment

Synopsis:

Description	Code	Problem	Comments
Strain Of Muscles/The Rotator Cuff Of Right Shoulder, Left	540611A		
Incomplete Rotator-Cuff Tear:right OF L Shoulder, Not Trauma	M75.117		
Lesion Of Ulnar Nerve, Left Upper Limb	C56.22		

Scott, Alvin Dean
61011 MIDB0292767 Loc 1808147315
#1619061 S7 y.o.M 1 SPO 121512915

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MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**H&P Notes (continued)**

PRINTED: 10/22/25 14:11:19 To: (RNS) (13724064062) 14:11:19 08/29/18 E1 Pg 6-9

08/29/2018

SCOTT, ALVIN (PHYSICIAN)

Plan

Procedure Coding:

Follow-Up:

Type	Rec'd Date	Provider Name	Notes
Physical	8/30/2018	WEY, JOHN	patient has no history of right or left shoulder pain after lifting heavy items. After left shoulder surgery, patient reports side tear supraspinatus, labrum, AC joint, arthroscopy. After right shoulder surgery, patient reports full thickness tear, anterior blood fraying of supraspinatus, less than 20% thickness, moderate to severe rotator cuff tear, full thickness, right glenohumeral arthritis, degenerative tear posterior capsule/rotator, small subacromial spur and thickening, acromioclavicular joint (CA) joint, right humerus. Recommended conservative treatment for right shoulder. Discharge instructions to outpatient with nurse and orthopedic for left. Dr. 1, partial rotator cuff tear left shoulder 2, supinator tear left shoulder 3, partial rotator cuff tear right shoulder Phys. 1, moderate rotator cuff tear left shoulder 2, full thickness arthroscopy, subacromial decompression 1, arthroscopic rotator cuff repair left 4, subacromial decompression right shoulder Time 2 hr Anesthesia <i>Wey 8/29/18</i>

Scott, Alvin Dean
 MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M
 516-1961-57 ext. 1000 CSN: 122512913

10/22/25 14:11:19 08/29/18 E1 Pg 6-9

Discharge Summary Note**Discharge Summary by Wey, John T., MD at 8/30/2018 1132**

Author: Wey, John T., MD

Filed: 08/30/18 1134

Editor: Wey, John T., MD (Physician)

Service: Orthopedics

Date of Service: 08/30/18 1132

Author Type: Physician

Status: Signed

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Discharge Summary Note (continued)****Outpatient Discharge Summary****BRIEF OVERVIEW**

Admission Date: 8/30/2018 Discharge Date: 8/30/2018

Admitting Provider: John T. Wey, MD

Discharge Provider: Wey, John T.

Primary Care Physician at Discharge: Unknown, Doctor None

Primary Discharge Diagnosis

Partial rotator cuff tear left and right shoulder, labral tear left shoulder

Discharge Disposition

Code Status at Discharge: full

Referrals and Follow-ups to Schedule**Lifting Restrictions (Specify)**

Maximum Weight to Lift: No Lifting

DETAILS OF HOSPITAL STAY**Presenting Problem/History of Present Illness**

Incomplete rotator cuff tear or rupture of left shoulder, not specified as traumatic [M75.112]

Superior glenoid labrum lesion of left shoulder, initial encounter [S43.432A]

Strain of muscle(s) and tendon(s) of the rotator cuff of right shoulder, initial encounter [S46.011A]

Operative Procedures Performed

Procedure(s):

ARTHROSCOPY SHOULDER--SUBACROMIAL DECOMPRESSION, LABRAL DEBRIDEMENT

SUBACROMIAL CORTISONE INJECTION--RIGHT SHOULDER

Physical Exam at Discharge

Discharge Condition: good

Heart Rate: 69

Resp: 18

BP: (I) 126/82

Temp: 97 °F (36.1 °C)

Weight: 159 lb 14.4 oz (72.5 kg)

Discharge Medications

Printed on 10/22/25 1:28 PM

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MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Discharge Summary Note (continued)**

Percocet, Zofran

Time spent on Discharge: 30 minutes or less

Electronically signed by Wey, John T., MD at 08/30/18 1134

Clinical Notes**Op Note****Wey, John T., MD at 8/30/2018 1128**

Author: Wey, John T., MD

Filed: 09/04/18 0752

Editor: Wey, John T., MD (Physician)

Service: Orthopedics

Date of Service: 08/30/18 1128

Author Type: Physician

Status: Signed

DATE OF PROCEDURE: 08/30/2018

PREOPERATIVE DIAGNOSIS:

1. Partial rotator cuff tear, left shoulder.
2. Superior labral tear, left shoulder.
3. Partial rotator cuff tear, right shoulder.

POSTOPERATIVE DIAGNOSES:

1. Partial rotator cuff tear, left shoulder.
2. Superior labral tear, left shoulder.
3. Partial rotator cuff tear, right shoulder.

NAME OF PROCEDURE:

1. Left shoulder arthroscopy, labral debridement and bursectomy.
2. Arthroscopic subacromial decompression, left shoulder.
3. Subacromial cortisone injection, right shoulder.

SURGEON: John Wey, MD

ANESTHESIOLOGIST: Kim Richmond, MD

ANESTHESIA: General and interscalene block.

PROCEDURE: The patient was identified and the operative sites marked. The patient received 1 gram of Ancef. A timeout was performed prior to starting surgery. 120 mg of Depo-Medrol was injected into the right subacromial space.

The left arm was then prepped and draped in the usual manner. Standard arthroscopic portals were made. Examination was made of the glenohumeral joint. There was no cartilage lesions. The patient did have degenerative tearing involving the anterior and superior labrum. The torn edges of the labrum were debrided with the ArthroCare wand. The biceps tendon and its anchor were intact. Shoulder was placed through range of motion and the rotator cuff examined and probed. There was tearing involving the most superficial fibers. These fibers were debrided to confirm the superficial nature of the tear.

Arthroscope was then inserted into the subacromial space. There was extensive bursitis. A bursectomy was carried out and the bursal surface of the rotator cuff tendon examined and probed. Once again, there was just superficial fraying of the rotator cuff tendon. This did not require a repair. Soft tissue from the underside acromion was excised to expose a downsloping acromion with a large lateral spur as well. The barrel bur was placed through the lateral portal to start the acromioplasty. This was extended to the level of the acromioclavicular joint. The bur was then placed through the posterior portal to further refine the acromioplasty. A flat undersurface of the acromion was obtained.

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) In Mrmc Or Main (continued)**Clinical Notes (continued)**

Subacromial space was then irrigated. Arthroscopic portals closed. Dressings were placed followed by a shoulder sling. An interscalene block was given by Dr. Richmond. The patient was taken in stable condition to the recovery room.

Dictated By: JOHN T. WEY, MD

D: 08/30/2018 11:28:07

T: 08/30/2018 11:46:53

Epic Job #: 1305882

Dictation Job #: 134781

JOHN T. WEY MD

Electronically signed by Wey, John T., MD at 09/04/18 0752

Labs**POCT Glucose (Final result)**Electronically signed by: **Interface, Results From Lab Instruments - To Beaker, Poct In** on 08/30/18 0845Status: **Completed**

Ordering user: Interface, Results From Lab Instruments - To Beaker, Poct In 08/30/18 0845

Authorized by: Unknown, Doctor

Ordering mode: Standard

Frequency: Routine Once 08/30/18 0846 - 1 occurrence

Class: Point Of Care

Quantity: 1

Lab status: Final result

Instance released by: Interface, Results From Lab Instruments - To Beaker, Poct In (auto-released) 8/30/2018 8:45 AM

Specimen Information

ID	Type	Source	Collected By
18RB-242P00094	Blood	Blood, Capillary	08/30/18 0843

POCT Glucose (Abnormal)

Resulted: 08/30/18 0845, Result status: Final result

Order status: Completed

Filed by: Lab, Background User 08/30/18 0845

Collected by: 08/30/18 0843

Resulting lab: MRMCM RBR LAB

CLIA number: 45D1088360

Components

Component	Value	Reference Range	Flag	Lab
Glucose Blood, POC	194	70 - 110 mg/dL	H ^	RBR

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1230000025 - RBR	MRMC RBR LAB	Unknown	2831 E PRESIDENT GEORGE BUSH HWY RICHARDSON TX 75082	03/09/16 1350 - Present

Glucose, random (Discontinued)Electronically signed by: **Richmond, Kim B, DO** on 08/30/18 1031Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

Frequency: Routine Once 08/30/18 1031 - 1 occurrence

Class: Lab Collect

Quantity: 1

Instance released by: Richmond, Kim B, DO (auto-released) 8/30/2018 10:31 AM

Discontinued by: Lab, Admin 11/13/19 1134 [Patient Discharge]

Order comments: FSG in PACU- call if < 75 or > 200

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MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) In Mrmc Or Main (continued)**Labs (continued)****Specimen Information**

ID	Type	Source	Collected By
—	Blood	Blood, Venous	—

POCT Glucose (Final result)Electronically signed by: **Interface, Results From Lab Instruments - To Beaker, Poct In** on 08/30/18 1146Status: **Completed**

Ordering user: Interface, Results From Lab Instruments - To Beaker, Poct In 08/30/18 1146

Authorized by: Unknown, Doctor

Ordering mode: Standard

Frequency: Routine Once 08/30/18 1147 - 1 occurrence

Class: Point Of Care

Quantity: 1

Lab status: Final result

Instance released by: Interface, Results From Lab Instruments - To Beaker, Poct In (auto-released) 8/30/2018 11:46 AM

Specimen Information

ID	Type	Source	Collected By
18RB-242P00137	Blood	Blood, Capillary	08/30/18 1144

POCT Glucose (Abnormal)

Resulted: 08/30/18 1146, Result status: Final result

Order status: Completed

Filed by: Lab, Background User 08/30/18 1146

Collected by: 08/30/18 1144

Resulting lab: MRMC RBR LAB

CLIA number: 45D1088360

Components

Component	Value	Reference Range	Flag	Lab
Glucose Blood, POC	281	70 - 110 mg/dL	H ^	RBR

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
1230000025 - RBR	MRMC RBR LAB	Unknown	2831 E PRESIDENT GEORGE BUSH HWY RICHARDSON TX 75082	03/09/16 1350 - Present

Other Orders**Medications****PROPOFOL 10 MG/ML INTRAVENOUS EMULSION (Active)**

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:

Ordering mode: Standard

Frequency: - Until Discontinued

Package: 63323-269-94

GLYCOPYRROLATE 0.2 MG/ML INJECTION SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:

Ordering mode: Standard

Frequency: - Until Discontinued

Package: 70121-1396-1

NEOSTIGMINE METHYLSULFATE 1 MG/ML INTRAVENOUS SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:

Ordering mode: Standard

Frequency: - Until Discontinued

Package: 63323-415-10

LIDOCAINE (PF) 100 MG/5 ML (2 %) INTRAVENOUS SYRINGE (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 76329-3390-1

ROCURONIUM 10 MG/ML INTRAVENOUS SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 0781-3220-75

FAMOTIDINE (PF) 20 MG/2 ML INTRAVENOUS SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 63323-739-16

ONDANSETRON HCL (PF) 4 MG/2 ML INJECTION SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 23155-196-31

EPHEDRINE SULFATE 50 MG/ML INJECTION WRAPPER (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 17478-515-00

CEFAZOLIN 1 GRAM SOLUTION FOR INJECTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 25021-101-67

METOCLOPRAMIDE 5 MG/ML INJECTION SOLUTION (Active)

Electronically signed by: on 08/31/18 1536

Status: **Active**

Ordering user:
Frequency: - Until Discontinued

Ordering mode: Standard
Package: 0409-3414-18

lactated Ringer's irrigation solution - ADS Override Pull (Completed)

Electronically signed by: Interface, Ads Dispense on 08/30/18 0742

Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0742

Ordering mode: Standard

Frequency: 08/30/18 0742 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

Medication comments: Shodi, Adrian: cabinet override

Package: 0338-0137-27

EPINEPHrine (ADRENALIN) 1 mg/mL (1 mL) injection - ADS Override Pull (Completed)

Electronically signed by: Interface, Ads Dispense on 08/30/18 0742

Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0742

Ordering mode: Standard

Frequency: 08/30/18 0742 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

COMMON SIDE EFFECTS: Dizziness, Nausea, Flushing and Anxiety

Medication comments: Shodi, Adrian: cabinet override

Package: 0409-7241-01

lactated Ringer's infusion (Discontinued)

Electronically signed by: Patton, Kasey G., RN on 08/29/18 1349

Status: **Discontinued**

Ordering user: Patton, Kasey G., RN 08/29/18 1349

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Transcribed from paper

Frequency: Routine Continuous 08/30/18 0815 - 08/30/18 1151

Class: Normal

Released by: Arthur, Kristin D., RN 08/30/18 0804

Discontinued by: Transfer Provider, Automatic 08/30/18 1151
[Patient Transfer]

Acknowledged: Arthur, Kristin D., RN 08/30/18 0804 for Placing Order

Package: 0338-0117-04

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)****lidocaine PF (XYLOCAINE) local injection 1 mg (Discontinued)**Electronically signed by: **Patton, Kasey G., RN on 08/29/18 1349**Status: **Discontinued**

Ordering user: Patton, Kasey G., RN 08/29/18 1349

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Transcribed from paper

PRN Comment: as needed

Frequency: Routine PRN 08/30/18 0804 - 08/30/18 1151

Class: Normal

Released by: Arthur, Kristin D., RN 08/30/18 0804

Discontinued by: Transfer Provider, Automatic 08/30/18 1151
[Patient Transfer]

Acknowledged: Arthur, Kristin D., RN 08/30/18 0804 for Placing Order

Package: 63323-492-27

lactated Ringer's infusion - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0818**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0818

Ordering mode: Standard

Frequency: 08/30/18 0818 - 1 occurrence

Admin instructions: Arthur, Kristin: cabinet override

Medication comments: Arthur, Kristin: cabinet override

Package: 0338-0117-04

lidocaine PF (XYLOCAINE) 10 mg/mL (1 %) local injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0830**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0830

Ordering mode: Standard

Frequency: 08/30/18 0830 - 1 occurrence

Admin instructions: Arthur, Kristin: cabinet override

Medication comments: Arthur, Kristin: cabinet override

Package: 63323-492-16

fentanyl PF (SUBLIMAZE) 50 mcg/mL injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0902**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0902

Ordering mode: Standard

Frequency: 08/30/18 0902 - 1 occurrence

Admin instructions: Richmond Md, Kim: cabinet override

COMMON SIDE EFFECTS: Shortness of breath, Drowsiness, Nausea/Vomiting, and

Constipation

Medication comments: Richmond Md, Kim: cabinet override

Package: 0409-9093-32

midazolam (VERSED) injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0903**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0903

Ordering mode: Standard

Frequency: 08/30/18 0903 - 1 occurrence

Admin instructions: Richmond Md, Kim: cabinet override

COMMON SIDE EFFECTS: Dizziness/Drowsiness, Confusion and Dry mouth

Medication comments: Richmond Md, Kim: cabinet override

Package: 0409-2305-17

acetaminophen (OFIRMEV) 1,000 mg/100 mL (10 mg/mL) infusion - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0903**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0903

Ordering mode: Standard

Frequency: 08/30/18 0903 - 1 occurrence

Admin instructions: Richmond Md, Kim: cabinet override

COMMON SIDE EFFECTS: Nausea/Vomiting, Weakness and Sleep disturbance

Medication comments: Richmond Md, Kim: cabinet override

Package: 43825-102-01

bupivacaine PF (MARCAINE) 0.5 % (5 mg/mL) injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0903**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0903

Ordering mode: Standard

Frequency: 08/30/18 0903 - 1 occurrence

Admin instructions: Richmond Md, Kim: cabinet override

COMMON SIDE EFFECTS: Injection site irritation, Sudden vision change,

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)**

Imbalance and Weakness

Medication comments: Richmond Md, Kim: cabinet override

Package: 0409-1162-02

methyIPREDNISolone acetate (DEPO-MEDROL) 40 mg/mL Injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 0934**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 0934

Ordering mode: Standard

Frequency: 08/30/18 0934 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

****NOT FOR IV USE!!****

Medication comments: Shodi, Adrian: cabinet override

Package: 0009-3073-01

lactated Ringer's irrigation solution (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1129**Status: **Discontinued**

Mode: Ordering in Verbal readback mode

Communicated by: Shodi, Adrian G., RN

Ordering user: Shodi, Adrian G., RN 08/30/18 1039

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Verbal readback

Frequency: Routine Continuous PRN 08/30/18 1020 - 08/30/18 1140

Class: Normal

Discontinued by: Shodi, Adrian G., RN 08/30/18 1140 [Patient Discharge]

Acknowledged: Shodi, Adrian G., RN 08/30/18 1039 for Placing Order

Package: 0338-0137-27

EPINEPHrine (ADRENALIN) Injection (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1129**Status: **Discontinued**

Mode: Ordering in Verbal readback mode

Communicated by: Shodi, Adrian G., RN

Ordering user: Shodi, Adrian G., RN 08/30/18 1040

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Verbal readback

Frequency: Routine PRN 08/30/18 1020 - 08/30/18 1140

Class: Normal

Discontinued by: Shodi, Adrian G., RN 08/30/18 1140 [Patient Discharge]

Acknowledged: Shodi, Adrian G., RN 08/30/18 1040 for Placing Order

Package: 0409-7241-01

methyIPREDNISolone acetate (DEPO-MEDROL) injection (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1129**Status: **Discontinued**

Mode: Ordering in Verbal readback mode

Communicated by: Shodi, Adrian G., RN

Ordering user: Shodi, Adrian G., RN 08/30/18 1049

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Verbal readback

Frequency: Routine PRN 08/30/18 1010 - 08/30/18 1140

Class: Normal

Discontinued by: Shodi, Adrian G., RN 08/30/18 1140 [Patient Discharge]

Acknowledged: Shodi, Adrian G., RN 08/30/18 1049 for Placing Order

Package: 0009-3073-01

EPINEPHrine (ADRENALIN) 1 mg/mL (1 mL) Injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 1102**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 1102

Ordering mode: Standard

Frequency: 08/30/18 1102 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

COMMON SIDE EFFECTS: Dizziness, Nausea, Flushing and Anxiety

Medication comments: Shodi, Adrian: cabinet override

Package: 0409-7241-01

EPINEPHrine (ADRENALIN) 1 mg/mL (1 mL) injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 1112**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 1112

Ordering mode: Standard

Frequency: 08/30/18 1112 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

COMMON SIDE EFFECTS: Dizziness, Nausea, Flushing and Anxiety

Medication comments: Shodi, Adrian: cabinet override

Package: 0409-7241-01

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)****lactated Ringer's irrigation solution - ADS Override Pull (Completed)**Electronically signed by: **Interface, Ads Dispense on 08/30/18 1144**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 1144

Ordering mode: Standard

Frequency: 08/30/18 1144 - 1 occurrence

Admin instructions: Shodi, Adrian: cabinet override

Medication comments: Shodi, Adrian: cabinet override

Package: 0338-0137-27

morphine injection solution 1 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: mild pain (1-3/10)

Frequency: STAT q5 min PRN 08/30/18 1157 - 08/30/18 1413

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413
[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Maximum total dose is 20mg while in PACU.

Morphine is option #1, if ineffective advance to dilaudid as ordered.

Package: 0409-1891-03

morphine injection solution 2 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: mod pain (4-6/10)

Frequency: STAT q5 min PRN 08/30/18 1157 - 08/30/18 1413

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413
[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Maximum total dose is 20mg while in PACU.

Morphine is option #1, if ineffective advance to dilaudid as ordered.

Package: 0409-1891-03

morphine injection solution 4 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: sev pain (7-10/10)

Frequency: STAT q5 min PRN 08/30/18 1157 - 08/30/18 1413

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413
[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Maximum total dose is 20mg while in PACU.

Morphine is option #1, if ineffective advance to dilaudid as ordered.

Package: 0409-1891-03

fentaNYL PF (SUBLIMAZE) injection 25 mcg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: sev pain (7-10/10)

Frequency: STAT q5 min PRN 08/30/18 1157 - 08/30/18 1413

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413
[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Maximum total dose is 250 mcg while in PACU.

COMMON SIDE EFFECTS: Shortness of breath, Drowsiness, Nausea/Vomiting, and

Constipation

Package: 0409-9093-35

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)****ondansetron PF (ZOFTRAN) injection 4 mg (Discontinued)**Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: nausea

Frequency: STAT q30 min PRN 08/30/18 1157 - 2 occurrences

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Ondansetron is considered option #1-if ineffective, progress to the next selected option for nausea.

Package: 23155-196-31

promethazine (PHENERGAN) IM injection 6.25 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: nausea

Frequency: STAT q30 min PRN 08/30/18 1157 - 4 occurrences

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Promethazine is considered option #3-if ineffective, progress to the next selected option for nausea.

Package: 0641-0955-21

labetalol (NORMODYNE,TRANDATE) injection 5 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN Comment: SBP>160 or DBP>80

Frequency: STAT q10 min PRN 08/30/18 1157 - 5 occurrences

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Hold for HR<60

Package: 0409-2267-20

hydrALAZINE (APRESOLINE) injection 5 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN Comment: SBP>160 or DBP>80

Frequency: STAT q10 min PRN 08/30/18 1157 - 4 occurrences

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Hold for HR<60

COMMON SIDE EFFECTS: Dizziness, Headache, Nausea, and Weight gain

Package: 67457-291-00

glycopyrrolate (ROBINUL) 0.2 mg/mL injection 0.2 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN Comment: HR less than 50

Frequency: STAT q10 min PRN 08/30/18 1157 - 08/30/18 1413

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: COMMON SIDE EFFECTS: Constipation, Dry mouth and Dizziness

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)**

Package: 0517-4601-25

meperidine PF (DEMEROL) injection 12.5 mg (Discontinued)Electronically signed by: **Richmond, Kim B, DO on 08/30/18 1031**Status: **Discontinued**

Ordering user: Richmond, Kim B, DO 08/30/18 1031

Ordering provider: Richmond, Kim B, DO

Authorized by: Richmond, Kim B, DO

Ordering mode: Standard

PRN reasons: shivering

Frequency: STAT q10 min PRN 08/30/18 1157 - 2 days

Class: Normal

Released by: Banthavong, Sibounheung, RN 08/30/18 1157

Discontinued by: Transfer Provider, Automatic 08/30/18 1413

[Patient Transfer]

Acknowledged: Banthavong, Sibounheung, RN 08/30/18 1157 for Placing Order

Admin instructions: Maximum total dose is 25 mg while in PACU.

Package: 0409-1176-30

ondansetron PF (ZOFTRAN) 4 mg/2 mL injection - ADS Override Pull (Completed)Electronically signed by: **Interface, Ads Dispense on 08/30/18 1221**Status: **Completed**

Ordering user: Interface, Ads Dispense 08/30/18 1221

Ordering mode: Standard

Frequency: 08/30/18 1221 - 1 occurrence

Admin instructions: BANTHAVONG, SIBOUNHEUNG: cabinet override

Medication comments: BANTHAVONG, SIBOUNHEUNG: cabinet override

Package: 23155-196-31

lactated Ringer's infusion (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1131**Status: **Discontinued**

Ordering user: Wey, John T., MD 08/30/18 1131

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Standard

Frequency: Routine Continuous 08/30/18 1345 - 08/30/18 1710

Class: Normal

Released by: Jarabata, Raquel A., RN 08/30/18 1330

Discontinued by: Discharge Provider, Automatic 08/30/18 1710

[Patient Discharge]

Acknowledged: Jarabata, Raquel A., RN 08/30/18 1330 for Placing Order

Package: 0338-0117-04

HYDROMORPHONE (DILAUDID) 1 mg/mL injection 0.4 mg (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1131**Status: **Discontinued**

Ordering user: Wey, John T., MD 08/30/18 1131

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Standard

PRN reasons: sev pain (7-10/10)

Frequency: Routine q2h PRN 08/30/18 1330 - 08/30/18 1710

Class: Normal

Released by: Jarabata, Raquel A., RN 08/30/18 1330

Discontinued by: Discharge Provider, Automatic 08/30/18 1710

[Patient Discharge]

Acknowledged: Jarabata, Raquel A., RN 08/30/18 1330 for Placing Order

Admin instructions: COMMON SIDE EFFECTS: Drowsiness, Nausea/Vomiting, Constipation and

Shortness of breath

Package: 0409-1283-31

oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet 2 tablet (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1131**Status: **Discontinued**

Ordering user: Wey, John T., MD 08/30/18 1131

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Standard

PRN reasons: mod pain (4-6/10)

PRN Comment: 2 tablets prior to discharge

Frequency: Routine q6h PRN 08/30/18 1330 - 08/30/18 1710

Class: Normal

Released by: Jarabata, Raquel A., RN 08/30/18 1330

Discontinued by: Discharge Provider, Automatic 08/30/18 1710

[Patient Discharge]

Acknowledged: Jarabata, Raquel A., RN 08/30/18 1330 for Placing Order

Package: 68084-355-11

ondansetron PF (ZOFTRAN) injection 4 mg (Discontinued)Electronically signed by: **Wey, John T., MD on 08/30/18 1131**Status: **Discontinued**

Ordering user: Wey, John T., MD 08/30/18 1131

Ordering provider: Wey, John T., MD

Authorized by: Wey, John T., MD

Ordering mode: Standard

MHS SERVICE AREA

Scott, Alvin Dean

MRN: MHD0399276, DOB: 5/10/1961, Legal Sex: M

Adm: 8/30/2018, D/C: 8/30/2018

08/30/2018 - Admission (Discharged) in Mrmc Or Main (continued)**Other Orders (continued)**

PRN reasons: nausea vomiting

Frequency: Routine q6h PRN 08/30/18 1330 - 08/30/18 1710

Released by: Jarabata, Raquel A., RN 08/30/18 1330

Class: Normal

Discontinued by: Discharge Provider, Automatic 08/30/18 1710
[Patient Discharge]

Acknowledged: Jarabata, Raquel A., RN 08/30/18 1330 for Placing Order

Package: 23155-196-31

ondansetron PF (ZOFran) 4 mg/2 mL injection - ADS Override Pull (Completed)

Electronically signed by: Interface, Ads Dispense on 08/30/18 1400

Status: Completed

Ordering user: Interface, Ads Dispense 08/30/18 1400

Ordering mode: Standard

Frequency: 08/30/18 1400 - 1 occurrence

Admin instructions: Jarabata, Raquel: cabinet override

Medication comments: Jarabata, Raquel: cabinet override

Package: 23155-196-31

oxyCODONE-acetaminophen (PERCOCET) 5-325 mg per tablet - ADS Override Pull (Completed)

Electronically signed by: Interface, Ads Dispense on 08/30/18 1401

Status: Completed

Ordering user: Interface, Ads Dispense 08/30/18 1401

Ordering mode: Standard

Frequency: 08/30/18 1401 - 1 occurrence

Admin instructions: Jarabata, Raquel: cabinet override

Medication comments: Jarabata, Raquel: cabinet override

Package: 68084-355-11

sodium chloride flush - ADS Override Pull (Completed)

Electronically signed by: Interface, Ads Dispense on 08/30/18 1402

Status: Completed

Ordering user: Interface, Ads Dispense 08/30/18 1402

Ordering mode: Standard

Frequency: 08/30/18 1402 - 1 occurrence

Admin instructions: Jarabata, Raquel: cabinet override

Medication comments: Jarabata, Raquel: cabinet override

Package: 8290306546

Patient: ALVIN SCOTT
Medical Record: #87169

HOPE - ADMISSION

Section A: Administrative Information

Patient / Admission Information

A0215. Site of service at admission 01. Patient's Home/Residence
A0220. Admission date 10/28/2025
A0500A. Patient first name ALVIN
A0500C. Patient last name SCOTT
A0550. Patient zip code 75154-8649
A0600A. Social Security Number 467-19-3962
A0600B. Patient Medicare/railroad insurance number 1U86AA7PN76
☒ N. Patient is not a Medicaid recipient
A0810. Sex 1. Male
A0900. Birthdate 5/10/1961

A1005. Ethnicity

☒ A1005A. No, not of Hispanic, Latino/a, or Spanish origin

A1010. Race

☒ A1010B. Black or African American

Language

A1110A. Preferred Language English
A1110B. Interpreter requested 0. No

Payor Information

☒ A1400A. Medicare (FFS)

Admitted From

A1805. Admitted From 01. Home/Community (pvt home/apt, board/care, alf, etc)

Living Arrangements

A1905 Living Arrangements 2. With others in the home (family, friends, paid caregiver)

Availability of Assistance

A1910 Availability of Assistance 1. Around-the-clock (24 hours a day with few exceptions)

Section F: Preferences

Was patient/responsible party asked about CPR 1. Yes
F2000B. Date asked about CPR 10/28/2025
Was patient/responsible party asked about treatements other than CPR 1. Yes
F2100B. Date asked about treatment other than CPR 10/28/2025
Was patient/responsible party asked about hospitalization 1. Yes
F2200B. Date asked about hospitalization 10/28/2025
Was patient/responsible party asked about spiritual/existential concerns 1. Yes
F3000B. Date asked about spiritual/existential concerns 10/28/2025

Section I: Active Diagnoses

I0010. Principal diagnosis

07. Heart Failure

Comorbidities and Co-existing Conditions

- ☒ I2900. Diabetes Mellitus (DM)
- ☒ I8005. Other Medical Condition

Section J: Health Conditions

Pain

- J0900A. Was patient screened for pain1. Yes
- J0900B. Date of first screening for pain10/28/2025
- J0900C. Patient's pain severity was2. Moderate
- J0900D. Type of standardized pain tool used2. Verbal descriptor
- J0905. Is pain an active problem for the patient?1. Yes
- J0910A. Was comprehensive pain assessment done1. Yes
- J0910B. Date of comprehensive pain assessment10/28/2025

Comprehensive pain assessment included:

- ☒ J0910C1. Location
- ☒ J0910C2. Severity
- ☒ J0910C3. Character
- ☒ J0910C4. Duration
- ☒ J0910C5. Frequency
- ☒ J0910C6. What relieves/worsens pain
- ☒ J0910C7. Effect on function or quality of life

J0915. Neuropathic Pain1. Yes

Respiratory Status

- J2030A. Was patient screened for shortness of breath1. Yes
- J2030B. Date of first screening for shortness of breath10/28/2025
- J2030C. Did screening indicate the patient had shortness of breath1. Yes
- J2040A. Was treatment for shortness of breath initiated2. Yes
- J2040B. Date treatment for shortness of breath initiated10/28/2025

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes

J2050B. Date of symptom impact screening10/28/2025

J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	2. Moderate	413680712
B. Shortness of breath	2. Moderate	413680712
C. Anxiety	1. Slight	1240413903
D. Nausea	0. Not at all	1240413903
E. Vomiting	0. Not at all	1240413903
F. Diarrhea	0. Not at all	1240413903
G. Constipation	0. Not at all	1240413903
H. Agitation	1. Slight	1240413903

J2052A. Was an in-person SFV completed?1. Yes

J2052B. Date of in-person SFV10/30/2025

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

- N0500A. Was scheduled opioid initiated or continued**

1. Yes
- N0500B. Date scheduled opioid initiated or continued** 10/28/2025
- N0510A. Was PRN opioid initiated or continued**

1. Yes
- N0510B. Date PRN opioid initiated or continued** 10/28/2025
- N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")** 2. Yes
- N0520B. Date bowel regimen initiated or continued** 10/28/2025

Section Z: Record Administration

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

10/30/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Patient: ALVIN SCOTT
Medical Record: #87169

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record2. HOPE Update Visit 1 (HUV1)

Patient / Admission Information

A0220. Admission date10/28/2025
A0500A. Patient first nameALVIN
A0500C. Patient last nameSCOTT
A0600A. Social Security Number467-19-3962
A0600B. Patient Medicare/railroad insurance number1U86AA7PN76

☒ N. Patient is not a Medicaid recipient

A0810. Sex1. Male
A0900. Birthdate5/10/1961

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes

J2050B. Date of symptom impact screening11/3/2025

J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	1. Slight	
E. Vomiting	1. Slight	
F. Diarrhea	1. Slight	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued11/3/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")2. Yes
N0520B. Date bowel regimen initiated or continued11/3/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed11/3/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/9/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record3. HOPE Update Visit 2 (HUV2)

Patient / Admission Information

A0220. Admission date10/28/2025
A0500A. Patient first nameALVIN
A0500C. Patient last nameSCOTT
A0600A. Social Security Number467-19-3962
A0600B. Patient Medicare/railroad insurance number1U86AA7PN76

☒ N. Patient is not a Medicaid recipient

A0810. Sex1. Male
A0900. Birthdate5/10/1961

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes

J2050B. Date of symptom impact screening11/13/2025

J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	1. Slight	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions?0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued11/13/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")2. Yes
N0520B. Date bowel regimen initiated or continued11/13/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed11/13/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/13/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Patient: ALVIN SCOTT
Medical Record: #87169

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record2. HOPE Update Visit 1 (HUV1)

Patient / Admission Information

A0220. Admission date10/28/2025
A0500A. Patient first nameALVIN
A0500C. Patient last nameSCOTT
A0600A. Social Security Number467-19-3962
A0600B. Patient Medicare/railroad insurance number1U86AA7PN76

☒ N. Patient is not a Medicaid recipient

A0810. Sex1. Male
A0900. Birthdate5/10/1961

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes

J2050B. Date of symptom impact screening11/3/2025

J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	1. Slight	
E. Vomiting	1. Slight	
F. Diarrhea	1. Slight	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued11/3/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes") 2. Yes
N0520B. Date bowel regimen initiated or continued11/3/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed11/3/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/15/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

HOSPICE AIDE CARE PLAN

Goals:

- ☒ Safe and effective personal care and hygiene
- ☒ Patient clean and comfortable
- ☒ Socialization/support
- ☒ Environment safe and clean

Update #1

Date: 10/28/2025 11:26 PM

Describe patient condition requiring HA interventions: Patient needs assistance with ADL

Patient Info

General Condition

- ☒ Symptoms managed well

Mental/Behavioral Status

- ☒ Withdrawn
- ☒ Confused
- ☒ Forgetful

Nutrition

- ☒ Diet
- Describe: As tolerated

Functional Limitations

- ☒ OOB with assist

Assistive Devices/Equipment Used During Patient Care

- ☒ Walker
- ☒ Wheelchair

Elimination

- ☒ Incontinent
 - ☒ Bladder
 - ☒ Bowel

Senses

- ☒ Hard of hearing
- ☒ Poor vision

Living Arrangements

- ☒ Lives in residential facility
- Describe: Home

Interventions

Pain Management

- ☒ Notify nurse if patient exhibits any verbal or non-verbal signs of pain or symptoms
- Special instructions: every visit

Bathing

- ☒ Shower
- Frequency: every visit

Hair Care

- ☒ Comb/brush
- Frequency: every visit
- ☒ Shampoo/style
- Frequency: Every Visit

Skin Care

- ☒ Inspect skin; check pressure areas
- Frequency: Every Visit
- ☒ Call nurse if new skin problems are noted
- Frequency: every visit
- ☒ Trim nails
 - ☒ Fingers
 - Frequency: Monthly
- ☒ Clean nails
 - ☒ Fingers
 - ☒ Toes
 - Frequency: every visit

Foot Care

- ☒ Bathe/inspect
- Frequency: Every Visit

Oral Care

- ☒ Inspect mouth. Call nurse if problems
- Frequency: Every Visit

☒ Assist with brushing teeth
Frequency: every visit

Elimination

☒ Incontinent care/diapers/briefs
Frequency: every visit

Home Management

☒ Clean patient's room/area
Frequency: every visit

Date: 10/28/2025 11:26 PM
Electronic Signature: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

COMFORT KIT ORDERS

All of the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring

- ☒ Morphine Sulfate, 100mg/5ml = 20 mg/ml, 0.25-1ml, Sublingual, Q1-2 hours prn, Pain/SOB - #120mL, 00054040441
- ☒ Lorazepam Liquid, 1.75mg/ml, 0.5-1 mg, PO/SL, q4 hrs prn, Anxiety/SOB - #30mL / RF: 5, 00054353244
- ☒ Atropine sulfate oral Soluntion, 0.75%, 1-2 drops, Take Sublingually, every 2 hours as needed, for excess secretions - #15mL / RF: 11
- ☒ APAP 650MG SUPPS, 650 mg, 1 suppository, unwrap and insert Rectally, Q 4 HRS PRN, Pain/Fever - #4 / RF:11 , 00927067812
- ☒ Bisacodyl suppository, 10 mg, 1 suppository, unwrap and insert Rectally, Q DAY PRN, Constipation - #3 / RF:11 , 00363010908
- ☒ Zofran ODT, 1 tablet, 4mg, dissolve PO/SL, Q 8 HRS PRN, nausea/vomiting - #6 / RF:11
- ☒ Senna, 8.6mg, 1 Tab, PO, Daily, Constipation
- ☒ Oxygen, na, 2-4 liters, intranasally, PRN, Shortness of breath

Ordering physician: DIEGO F RESTREPO MD
Date: 10/28/2025

Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 10/28/2025
Ordering Physician: Restepo
Reviewing Physician: DIEGO F RESTREPO MD
Medication: furosemide 20 MG Oral Tablet
Unit strength: 20 mg
Dose: 20mg
Form: Oral Tablet
NDC: 00054429725
Route: po
Frequency: Once daily
Indication: diuretics
Associated diagnosis: I25.10 - Athscl heart disease of native coronary artery w/o ang pctrs (Primary)
Days Supply: 15
Is medication covered? Yes

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 10/28/2025
Ordering Physician: Dr. Restrepo
Reviewing Physician: DIEGO F RESTREPO MD
Medication: acetaminophen 325 MG / hydrocodone bitartrate 10 MG Oral Tablet [Norco]
Unit strength: 325-10 mg
Dose: 1 tablet
Form: Oral Tablet
NDC: 52959053340
Route: Oral Pill
Frequency: Four times daily
Indication: Pain
Days Supply: 15
Is medication covered? Yes

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

MULTIPLE MEDICATIONS FORM

Multiple Medications Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Medications:

Start Date	Discontinue Date	Ordering Physician	Reviewing Physician	Medication	Unit strength	Dose	Form	NDC	Route	Frequency	Indication	Associated diagnosis	Days Supply	Is medication covered by hospice?	If not covered, reason medication is not covered
10/28/2025		Dr Restrepo	DIEGO F RESTREPO MD	24 HR glipizide 10 MG Extended Release Oral Tablet	10 mg	1tablet	Extended Release Oral Tablet	00247216030	Oral Pill	Once daily	DM		30	No	
10/28/2025		Dr Restrepo	DIEGO F RESTREPO MD	esomeprazole 20 MG Delayed Release Oral Capsule [Nexium]	20 mg	1 tablet	Delayed Release Oral Capsule	00186502031	po	Once daily	GERD		15	Yes	

Date: 10/28/2025

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

MULTIPLE MEDICATIONS FORM

Multiple Medications Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Medications:

Start Date	Discontinue Date	Ordering Physician	Reviewing Physician	Medication	Unit strength	Dose	Form	NDC	Route	Frequency	Indication	Associated diagnosis	Days Supply	Is medication covered by hospice?	If not covered, reason medication is not covered
11/14/2025	11/24/2025	Restrepo	DIEGO F RESTREPO MD	amoxicillin 875 MG / clavulanate 125 MG Oral Tablet [Augmentin]	875-125 mg	1tablet	Oral Tablet	52959047810	Oral Pill	Twice daily for10 days	URI		10	Yes	
11/14/2025		Restrepo	DIEGO F RESTREPO MD	Promethazine	6.25/5ml	5 ml			Po	Every 4-6 hours prn	Cough / allergies		15	Yes	
11/14/2025		Restrepo	DIEGO F RESTREPO MD	melatonin 3 MG Oral Tablet	3 mg	1 tablet	Oral Tablet	00761022640	po	Give once at bedtime	insomnia		15	Yes	

Date: 11/14/2025

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

INFECTION INCIDENT REPORT

INFECTION INCIDENT REPORT

Type of infection: Respiratory Infection
Diagnostic method: Presumed by MD based on signs and symptoms

Symptoms Displayed by Patient:

- ☒ Chills
- ☒ Drainage
- ☒ New or increased cough

Treatment prescribed? Yes
Date: 11/14/2025
List treatment: Augmentin 875/125mg bid x 10 days
Prescribing physician: DIEGO F RESTREPO MD
Patient/Caregiver notified of order? Yes
Date/Time notified: 11/14/2025 12:00:00 PM

Date: 11/14/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

COVID-19 ASSESSMENT

COVID-19 ASSESSMENT

This assessment tool supports you in identifying patients at risk for COVID-19 and helps guide clinicians in clinical decision making.

Symptoms

Note: Symptoms may appear 2-14 days after exposure.

☒ None

Travel History

***Updated information on affected countries can be found at: <https://www.cdc.gov/coronavirus/2019-ncov/travelers/index.html>*

Action Taken

☒ None indicated

***Please refer to the CDC COVID-19 website for ongoing updates and guidance: <https://www.cdc.gov/coronavirus/2019-ncov>*

COVID-19 TESTING

Patient testing completed? No

Date: 10/28/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

LCD - CARDIAC

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Recurrent or intractable infection; pneumonia, sepsis, UTI
- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss
- ☒ Dysphagia leading to recurrent aspiration and/or decreased oral intake documented by decreasing food portion consumption

Worsening of Symptoms

- ☒ Dyspnea with increasing respiratory rate
- ☒ Nausea, vomiting poorly responsive to treatment
- ☒ Intractable diarrhea

Worsening of Signs

- ☒ Edema
- ☒ Weakness
- ☒ Change in level of consciousness

- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs
- ☒ Increased ER visits, hospitalizations, or physician visits related to the hospice primary diagnosis

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ Ischemic heart disease
- ☒ Dementia

Cardiac

NEW YORK HEART ASSOCIATION CLASS (Heart disease)

- ☒ Class III (Moderate) Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes fatigue, palpitation, or dyspnea.

HEART DISEASE

Patients should meet criteria in numbers 1 and 2. Supporting documentation is found in no. 3.

- ☒ 1: Patient has already been optimally treated with vasodilators unless contraindicated AND is not a candidate for surgery or has refused surgery

3: Any of the following will help support the diagnosis but is not required:

- ☒ **Treatment resistant symptomatic supraventricular or ventricular arrhythmias.**
- ☒ **History of unexplained syncope**

Date: 10/28/2025 12:00:00 AM

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

LCD - GENERAL DECLINE IN CLINICAL STATUS

LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

Worsening of Clinical Status

- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss

Worsening of Signs

- ☒ Edema
- ☒ Weakness

- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs

Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ CHF
- ☒ Diabetes Mellitus
- ☒ Dementia

Date: 10/28/2025 12:01:00 AM

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

LVN NURSE VISIT

LVN NURSE COMPREHENSIVE VISIT

Date/Time in: 10/29/2025 3:12:00 PM
Date/Time out: 10/29/2025 4:02:00 PM
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 98.1

• Respiratory Rate

Respiratory rate (respirations per minute): 16
☒ Room air

• Heart Rate

Type of heart rate: Regular
Apical pulse: 103
Radial pulse: 103

• Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication
☒ Sitting
Systolic blood pressure: 145
Diastolic blood pressure: 91

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Alert
☒ Oriented

Current Behavioral Status

☒ Appropriate
☒ Cooperative

Speech Status

☒ Clear
☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 3
New, controlled, or uncontrolled? Controlled
Location/description of pain: back

Neurosensory

☒ Restless sleep

Cardiovascular

☒ Capillary refill > 3 seconds

Nutrition

☒ Patient satisfied with intake

Percent of meals eaten: 100

GI Elimination

Date of last bowel movement: 10/29/2025
Bowel Pattern regular
Bowel sounds, # of quads: 4

☒ WNL

☒ Abdomen: Soft

GU Elimination

Urine output: Good
Urine color: yellow

☒ Clear

Respiratory

☒ Clear

Describe location, inspiratory/expiratory: all lobes
Oxygen saturation percentage: 97

Mobility

☒ Ambulatory

Steady/unsteady gait: Steady gait

☒ Poor endurance

Skin

Skin temperature: warm
Skin turgor: good

Psychosocial Status

Psychosocial/emotional status: calm
Anticipatory grief/bereavement status: moderate

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Instructed on policies and procedures related to use and disposal of controlled drugs in the home

- ☒ Patient
- ☒ Family
- ☒ Caregiver

• Teaching provided during visit

• Method(s)

- ☒ Verbal instruction

• Outcome(s)

- ☒ Verbalized understanding

• Instructed on infection control procedures

- ☒ Patient
- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Family reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Additional Information/Comments: 64 y/o male admitted to hospice services with the primary I25.10 - Athscl heart disease of native coronary artery w/o ang pctrs patient lives in home with family, alert orient to self, place, time situation, pain 4/10 to lower back controlled by pain medication, head to toe sin assessment skin intact breath sounds even unlabor, clear all lobes, no edema, patient walks, blood pressure slightly elevated, encourage to take all medications as prescribed he v erbally understands.

Date: 10/29/2025

Electronically signed by: LATANDRA JONES LVN

Patient: ALVIN SCOTT
Medical Record: #87169

SFV ABRIDGED NURSING VISIT NOTE

Symptom Follow-Up Visit

Date/Time In: 10/30/2025 9:00:00 AM
Date/Time Out: 10/30/2025 9:30:00 AM
Type of visit: PRN
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: Follow up visit on moderate pain 4/10 rating per lvn visit , patient utilizing Norco 10/325 mg as needed for pain to lower back , shoulders , and bilateral legs . On self administration patient rates best pain 1/10, bearable pain 3-4/10, worst pain 8/10. Current plan of care is effective and controlling all symptoms . Plan of care on going with adjustments to be made by hospice MD as needed . Patient is compliant with medication regimen and in agreement with plan of care .
This is a follow-up visit for: HUV 1

Symptom Tracking

Was a symptom impact screening completed? 1. Yes

A. Pain	1. Slight
B. Shortness of breath	0. Not at all
C. Anxiety	0. Not at all
D. Nausea	0. Not at all
E. Vomiting	0. Not at all
F. Diarrhea	0. Not at all
G. Constipation	0. Not at all
H. Agitation	0. Not at all

Last bowel movement: 10/30/2025
Pain: 1/10
PRN medication usage last 24 hours: Norco 10/325 mg as needed x2 doses
Need medication refills: Yes
Discussion with caregiver/POA/facility staff: Patient is self POA has caregiver , discussed sx management and when to call hospice . Further education to be provided on disease process . Patient voices understanding at this time , ongoing education each visit .
Date: 10/30/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

HOPE UPDATE VISIT

Date

Reason for Visit

At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?

Was a symptom impact screening completed?

Date of symptom impact screening

Pain

Shortness of breath

Anxiety

Nausea

Vomiting

Diarrhea

Constipation

Agitation

11/3/2025

2. HOPE Update Visit 1 (HUV1)

0. No

1. Yes

11/3/2025

1. Slight

1. Slight

1. Slight

1. Slight

1. Slight

1. Slight

0. Not at all

0. Not at all

Skin Conditions

Does the patient have one or more skin conditions?

0. No

Medications

Was scheduled opioid initiated or continued?

Was PRN opioid initiated or continued?

Date PRN opioid initiated or continued

Was bowel regimen initiated or continued?

Date bowel regimen initiated or continued

0. No

1. Yes

11/3/2025

2. Yes

11/3/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:11/3/2025 2:30:00 PM

Date/Time out:11/3/2025 3:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:none

Patient/Family/Caregiver's primary concern during this visit:none

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 98.1
- Respiratory Rate

Respiratory rate (respirations per minute): 18

☒ Room air
- Heart Rate

Type of heart rate: Regular

Apical pulse: 100
- Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication

☒ Sitting

Systolic blood pressure: 130

Diastolic blood pressure: 80
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Garbled
- ☒ Slurred

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: generalized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Syncope
- ☒ Capillary refill > 3 seconds
- ☒ Edema

Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
- Percent of meals eaten: 50
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 11/3/2025
Bowel Pattern daily
Bowel sounds, # of quads: 4

- ☒ WNL

Describe: times 4

- ☒ Abdomen: Soft
- ☒ Incontinence

GU Elimination

Urine output: Fair

- ☒ Frequency

Describe: clear

- ☒ Clear
- ☒ Incontinent

Respiratory

- ☒ Diminished
- Describe location, inspiratory/expiratory: lower extremities
- ☒ Dyspnea at rest
- ☒ Dyspnea with exertion

Mobility

- ☒ Ambulatory
- Steady/unsteady gait: Steady gait
- ☒ Poor endurance
- ☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

Psychosocial Status

Psychosocial/emotional status: stable

Anticipatory grief/bereavement status: normal

HOPE Update Visit

Reason for Visit	2. HOPE Update Visit 1 (HUV1)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	11/3/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No

Was PRN opioid initiated or continued? 1. Yes

Date PRN opioid initiated or continued 11/3/2025

Was bowel regimen initiated or continued? 2. Yes

Date bowel regimen initiated or continued 11/3/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):

- Hospice philosophy and goals of care
- Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
- Plan of care, visit frequency, and 24/7 on-call availability
- Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
- Use, purpose, and possible side effects of current medications and comfort kit
- Safety and fall precautions in the home/facility
- When to call hospice vs. 911
- Patient and family rights, responsibilities, and informed consent
- Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family:

Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 11/3/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/4/2025 10:00:00 AM

Date/Time out:11/4/2025 10:30:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 98
- Respiratory Rate

Respiratory rate (respirations per minute): 18

☒ Room air

Oxygen liters per minute:96
- Heart Rate

Type of heart rate: Regular

Radial pulse:82
- Blood Pressure

☒ Sitting

Systolic blood pressure: 136

Diastolic blood pressure: 88
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled
Location/description of pain: None reported

Cardiovascular

☒ Edema
Location: BLE
Rating: +2

Nutrition

☒ Changes in appetite
☒ Appetite is a problem
Percent of meals eaten: 50
☒ Indigestion/heartburn

GI Elimination

Date of last bowel movement: 11/4/2025
☒ Incontinence

GU Elimination

☒ Incontinent

Respiratory

☒ Dyspnea with exertion
Oxygen saturation percentage: 96

Mobility

☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Normal
☒ Pale

Psychosocial Status

Psychosocial/emotional status: Calm

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Patient was educated on energy conservation techniques, including pacing activities, taking frequent rest breaks, and using assistive devices as needed to minimize exertion. Discussed the importance of elevating lower extremities when sitting or lying down to help reduce edema. Reinforced the need for a low-sodium diet to assist with fluid management and heart health. Encouraged the patient to monitor for any increased swelling, weight gain, or worsening shortness of breath and to report these symptoms promptly. Additionally, education was provided on maintaining adequate nutritional intake through small, frequent meals to help maintain energy levels and overall strength.

Additional Information/Comments: Alvin Scott is a 64-year-old patient with a history of atherosclerotic heart disease. He is alert and oriented to person, place, and time (AAOx3). The patient presents with generalized weakness and is able to perform 3 out of 6 activities of daily living (ADLs) independently. He is incontinent of bladder and reports a fair appetite, consuming approximately 50% of small meals. He is sleeping up to 8 hours nightly. During ambulation, the patient experiences noticeable shortness of breath, consistent with decreased endurance related to his cardiac condition. Bilateral lower extremities exhibit 2+ pitting edema, suggesting fluid retention possibly related to heart disease. The patient was observed resting comfortably following care, with oxygen saturation noted at 96% on room air.

Date: 11/4/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:11/13/2025 12:00:00 PM

Date/Time out:11/13/2025 1:15:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:none

Patient/Family/Caregiver's primary concern during this visit:none

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 98
- Respiratory Rate

Respiratory rate (respirations per minute): 16
- Heart Rate

Type of heart rate: Regular

Radial pulse: 82
- Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication

☒ Sitting

Systolic blood pressure: 132

Diastolic blood pressure: 88
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 2
New, controlled, or uncontrolled? Controlled
Location/description of pain: generalized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Syncope
- ☒ Capillary refill > 3 seconds
- ☒ Edema
Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
Percent of meals eaten: 50
- ☒ Patent
- ☒ Intact

GI Elimination

Date of last bowel movement: 11/13/2025
Bowel Pattern daily
Bowel sounds, # of quads: 4
☒ WNL
Describe: times 4
☒ Abdomen: Soft
☒ Incontinence

GU Elimination

Urine output: Fair
☒ Frequency
Describe: clear
☒ Clear
☒ Incontinent

Respiratory

- ☒ Diminished

Describe location, inspiratory/expiratory: lower extremities

- ☒ Dyspnea at rest
- ☒ Dyspnea with exertion

Mobility

- ☒ Ambulatory
- Steady/unsteady gait: Steady gait
- ☒ Poor endurance
- ☒ Weakness/fatigue

Skin

Skin temperature: warm
Skin turgor: poor

Psychosocial Status

Psychosocial/emotional status: stable
Anticipatory grief/bereavement status: normal

HOPE Update Visit

Reason for Visit	3. HOPE Update Visit 2 (HUV2)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	11/13/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 11/13/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 11/13/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):

- Hospice philosophy and goals of care
- Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
- Plan of care, visit frequency, and 24/7 on-call availability
- Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
- Use, purpose, and possible side effects of current medications and comfort kit
- Safety and fall precautions in the home/facility
- When to call hospice vs. 911
- Patient and family rights, responsibilities, and informed consent
- Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family:

Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
- ☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 11/13/2025

Electronically signed by: TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/14/2025 12:00:00 PM

Date/Time out:11/14/2025 12:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 99
- Respiratory Rate

Respiratory rate (respirations per minute): 16

☒ Room air
- Heart Rate

Radial pulse: 114
- Blood Pressure

Hypertension or hypotension? Hypertension

☒ Sitting

Systolic blood pressure: 132

Diastolic blood pressure: 80
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Comments related to vital signs: Currently not taking blood pressure medications due to refusal . Only utilizing Lasix .

Current Mental Status

- ☒ Alert
- ☒ Oriented

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: Lower back and both legs

Neurosensory

☒ Restless sleep

Nutrition

☒ Patient satisfied with intake
Percent of meals eaten: 50
☒ Indigestion/heartburn

GI Elimination

Date of last bowel movement: 11/14/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft

GU Elimination

Urine output: Good
Urine color: Yellow
☒ Clear

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: All lobes
☒ Diminished
Describe location, inspiratory/expiratory: Bases
Oxygen saturation percentage: 95

Mobility

☒ Ambulatory
Steady/unsteady gait: Steady gait
☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Fair

Psychosocial Status

Psychosocial/emotional status: Calm

Anticipatory grief/bereavement status: Moderate

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Describe:

Drug Name & Strength	Symptom Treated	# of Doses	Effectiveness (0-10)
Norco 10/325mg	pain to lower back	1	10

Does patient require refill of any medication? No

Communication

- Teaching provided during visit
 - Topic(s) discussed (include topic(s) and person(s) receiving instruction): Disease process
 - Method(s)
 - ☒ Verbal instruction
 - Outcome(s)
 - ☒ Verbalized understanding

Communication with other team members (include name, title, date, time, and topic(s) discussed): Dr Restrepo, see below

☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

Plan for next visit: Follow up on sx

Additional Information/Comments: 64 year old black male with hospice dx Atherosclerotic heart disease. Alert and oriented x 3. Clear speech .Patient resides in a home setting . Has paid caregiver to assist with light house work , cooking , ADLs, laundry . Patient requires 4/6 ADLS . Continent at this time of bowel and bladder . Self feeds up to 50% of small meals . Focus of today's visit : patient has complaints of yellow sputum , pain upon coughing , running a low grade temperature. Voiced concerns of ongoing pneumonia without treatment due to past experiences . Patient also complained of restless sleep . New orders obtained from hospice Md for patient to begin Augmentin 875/125 mg bid x 10 days , melatonin 3 mg QHS , promethazine 6.25/5ml q 4 hours prn not to exceed 30ml in 24 hours . Education provided on signs of adverse reactions to new orders and when to call hospice . Follow up visit scheduled for tomorrow to assess effectiveness of treatment . Plan of care ongoing.

Date: 11/14/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

SFV ABRIDGED NURSING VISIT NOTE

Symptom Follow-Up Visit

Date/Time In: 11/15/2025 1:00:00 PM
Date/Time Out: 11/15/2025 1:30:00 PM
Type of visit: PRN
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: Patient had follow up visit for Augmentin 875/125 mg bid x 10 days, no adverse reaction noted, patient resting comfortably. Will continue routine and supportive care.
This is a follow-up visit for: HUV 2

Symptom Tracking

Was a symptom impact screening completed? 1. Yes

A. Pain	1. Slight
B. Shortness of breath	0. Not at all
C. Anxiety	1. Slight
D. Nausea	0. Not at all
E. Vomiting	0. Not at all
F. Diarrhea	0. Not at all
G. Constipation	0. Not at all
H. Agitation	1. Slight

Last bowel movement: 11/14/2025
Pain: 1
PRN medication usage last 24 hours: no
Need medication refills: No
Discussion with caregiver/POA/facility staff: Discussed with caregiver adverse reactions to new medications.
Date: 11/15/2025

Electronically signed by: TANEKA DAVIS LVN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/17/2025 11:00:00 AM

Date/Time out:11/17/2025 11:30:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 98.4

• Respiratory Rate

Respiratory rate (respirations per minute): 19
☒ Room air
Oxygen liters per minute: 96

• Heart Rate

Type of heart rate: Regular
Radial pulse: 80

• Blood Pressure

Hypertension or hypotension? Hypertension
☒ Sitting
Systolic blood pressure: 126
Diastolic blood pressure: 74

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Alert
☒ Oriented

Current Behavioral Status

☒ Appropriate
☒ Cooperative

Speech Status

☒ Clear
☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: Back, shoulders, legs.

Nutrition

☒ Patient satisfied with intake
Percent of meals eaten: 50

GI Elimination

Date of last bowel movement: 11/17/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft

GU Elimination

Urine output: Good
Urine color: Yellow
☒ Clear
Other: Continent of bladder

Respiratory

☒ Clear
Describe location, inspiratory/expiratory: All lobes
☒ Diminished
Describe location, inspiratory/expiratory: Bases
Oxygen saturation percentage: 96

Mobility

☒ Ambulatory
Steady/unsteady gait: Unsteady gait
☒ Poor endurance
☒ Weakness/fatigue

Skin

Skin temperature: Warm
Skin turgor: Fair

RN Supervisory Visit (1)

Hospice Caregiver Discipline Homemaker
Present/not present Not present
• Interview With

☒ Patient

- *Follows assignment/plan of care*

☒ Yes

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

• Patient's room is clean and tidy

☒ Yes

• Patient's linens are fresh and clean

☒ Yes

• Assignment sheet/POC reviewed

☒ Yes

• Assignment sheet/POC updated

☒ Yes

Comments: Continue POC

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Describe:	Drug Name & Strength	Symptom Treated	# of Doses	Effectiveness (0-10)
	Norco 10/325mg	Pain to back, shoulders, legs.	1	10

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Teaching focused on medication safety, including monitoring for adverse reactions to current medications and reporting any new symptoms promptly. Caregivers were instructed to continue supporting the patient with 4 out of 6 ADLs, encouraging safe mobility and assisting with transfers as needed. Reinforcement was provided by offering small, manageable meals and promoting hydration, as the patient is self-feeding up to 50% of small meals. Education included continued awareness of pain patterns, effective use of prescribed medications, and observing for changes in bowel or bladder habits, alertness, or overall comfort. Emphasis was also placed on maintaining a calm, supportive environment to promote safety and well-being.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Additional Information/Comments: The patient is a 64-year-old black male with a hospice diagnosis of Atherosclerotic heart disease. The patient has shown no adverse reaction to Augmentin and is using promethazine 6.25/5 mL for allergies and nausea with good tolerance. He remains continent of bowel and bladder and requires assistance with 4 of 6 ADLs. Norco 10/325 mg continues to be effective in managing pain in the back, shoulders, and legs, and is taken routinely every four hours, providing good relief. The patient is alert and oriented x3, engages appropriately, and self-feeds approximately 50% of small meals. The current plan of care remains effective, and no new concerns were identified during the visit.

Date: 11/17/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in:

11/19/2025 11:00:00 AM

Date/Time out:

11/19/2025 11:05:00 AM

Purpose of progress note:

IDG Narrative

IDG Narrative

Ongoing treatment for uri initiated 11/14/25: Augmentin 875-125 mg bid x 10 days

Date: 11/19/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:11/26/2025 12:50:00 PM

Date/Time out:11/26/2025 1:25:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 97.3

• Respiratory Rate

Respiratory rate (respirations per minute): 18
☒ Room air
Oxygen liters per minute: 95

• Heart Rate

Type of heart rate: Regular

• Blood Pressure

☒ Sitting
Systolic blood pressure: 132
Diastolic blood pressure: 74

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Alert
☒ Oriented

Current Behavioral Status

☒ Appropriate
☒ Cooperative

Speech Status

☒ Clear
☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: Back, shoulders, legs pain.

Nutrition

☒ Patient satisfied with intake
Percent of meals eaten: 50

GI Elimination

Date of last bowel movement: 11/26/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft

GU Elimination

Urine output: Good
Urine color: Yellow
☒ Clear

Respiratory

☒ Cough
Type of cough: Productive
Sputum color: Clear
Oxygen saturation percentage: 95

Mobility

☒ Ambulatory
Steady/unsteady gait: Unsteady gait
☒ Poor endurance
☒ Weakness/fatigue

RN Supervisory Visit (1)

Hospice Caregiver Discipline Homemaker
Present/not present Not present

• Interview With
☒ Patient

• Follows assignment/plan of care
☒ Yes

• Patient's needs are being met
☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

- *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

Comments: Continue POC

RN Supervisory Visit (2)

Hospice Caregiver Discipline	LPN
Hospice caregiver name:	Taneeka
Present/not present:	Not present

• *Interview With*

☒ Patient

• *Follows assignment/plan of care*

☒ Yes

• *Patient's needs are being met*

☒ Yes

• *Patient/family/CG satisfied with care provided*

☒ Yes

• *Treats patient with dignity and respect*

☒ Yes

• *Complies with infection control procedures*

☒ Yes

• *Reports changes in patient's condition to RN*

☒ Yes

• *Punctual: notifies office if late*

☒ Yes

• *Dresses properly*

☒ Yes

• *Interacts well with patient/family/CG*

☒ Yes

• *Provides care in a safe manner*

☒ Yes

• *Arrives on time or calls if late*

☒ Yes

• *Competent with assigned tasks*

Yes



Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Describe:	Drug Name & Strength	Symptom Treated	# of Doses	Effectiveness (0-10)
	Norco 10/325 mg	Pain	1	10

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Education was given on monitoring pain levels and using medication consistently to maintain comfort. Reinforcement was provided on strategies to reduce fatigue during ADLs, including pacing and resting between tasks. Guidance was offered on when to report changes in cough, sputum color, or breathing patterns. Caregiver support was acknowledged, and instruction was provided on safe hygiene assistance and maintaining the patient's dignity during care. Continuous communication with the hospice team was encouraged should any symptoms worsen or new concerns arise.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: The patient is a 64-year-old Black male receiving hospice care for atherosclerotic heart disease. He remains continent of both bowel and bladder and currently needs help with 3/6 ADLs .due to ongoing weakness. His prescribed Norco 10/325 mg every four hours continues to provide reliable relief for discomfort in his back, shoulders, and legs. Denies constipation with use . Prn senna available .He was alert, oriented, and interacted appropriately during the visit. He is eating about half of his small meals and can feed himself. The patient reports a persistent cough with occasional clear sputum; he notes improvement since completing antibiotic therapy. Because of his decreased strength, he has a paid caregiver helping with hygiene and daily personal care decreasing risk of injury . Reviewed POC with pt , current POC effective .

Date: 11/26/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in: 11/30/2025 12:02:00 AM
Date/Time out: 11/30/2025 12:03:00 AM
Purpose of progress note: IDG Narrative
IDG Narrative Continue plan of care

Date: 11/30/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

LVN NURSE VISIT

LVN NURSE COMPREHENSIVE VISIT

Date/Time in: 12/4/2025 4:15:00 PM
Date/Time out: 12/4/2025 4:45:00 PM
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- *Temperature*
Type of temperature taken: Temporal:
Temporal temperature: 97.5
- *Respiratory Rate*
Respiratory rate (respirations per minute): 18
- *Heart Rate*
Radial pulse: 87
- *Blood Pressure*
☒ Sitting
Systolic blood pressure: 136
Diastolic blood pressure: 93
- *Pedal Pulses*
☒ Present
☒ Right
☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale

Pain Rating: 3
New, controlled, or uncontrolled? Controlled

GI Elimination

Date of last bowel movement: 12/4/2025
Bowel Pattern regular
Bowel sounds, # of quads: 4

Respiratory

- ☒ Dyspnea at rest
- ☒ Dyspnea with exertion

Mobility

- ☒ Poor endurance
- ☒ Weakness/fatigue

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Instructed on policies and procedures related of use and disposal of controlled drugs in the home

- ☒ Patient

• Teaching provided during visit

• Method(s)

- ☒ Verbal instruction

• Outcome(s)

- ☒ Verbalized understanding

• Instructed on infection control procedures

- ☒ Patient

- ☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

- ☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: Patient complained fatigue, patient educated on pacing himself throughout the day to prevent fatigue. Patient was encouraged to takes naps whenever felt overly exerted for fatigue prevention. Patient left resting on sofa, no signs of distress noted, will continue routine and supportive care. (See Modifications 1)

Date: 12/5/2025

Electronically signed by: TANEEKA DAVIS LVN

Form Modifications

Number	Field	Previous Value	Modified On	Modified By	Reason
1	Additional Information/Comments:	Patient complained fatigue, patient educated on pacing himself throughout the day to prevent fatigue. Patient was encouraged to takes naps whenever felt overly exerted for fatigue prevention	12/10/2025 11:42 PM	TANEEKA DAVIS	Changed due to change in linked document

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:12/9/2025 3:35:00 PM

Date/Time out:12/9/2025 4:05:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 97.4
- Respiratory Rate

Respiratory rate (respirations per minute): 18

☒ Room air
- Heart Rate

Type of heart rate: Regular

Radial pulse: 88
- Blood Pressure

☒ Sitting

Systolic blood pressure: 136

Diastolic blood pressure: 84
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Nutrition

☒ **Patient satisfied with intake**
Percent of meals eaten: 75

GI Elimination

Date of last bowel movement: 12/9/2025
Bowel Pattern Regular
☒ **WNL**
☒ **Abdomen: Soft**

GU Elimination

Urine output: Good
☒ **Clear**

Respiratory

☒ **Clear**
Describe location, inspiratory/expiratory: All lobes
☒ **Dyspnea at rest**
☒ **Dyspnea with exertion**
Oxygen saturation percentage: 96

Mobility

☒ **Ambulatory**
Steady/unsteady gait: Steady gait
☒ **Poor endurance**
☒ **Weakness/fatigue**

Skin

Skin temperature: Warm
Skin turgor: Fair

RN Supervisory Visit (1)

Hospice Caregiver Discipline Homemaker
Present/not present Not present

• *Interview With*

☒ **Patient**
☒ **PCG**

• *Follows assignment/plan of care*

☒ **Yes**

- *Patient's needs are being met*

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

• *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

RN Supervisory Visit (2)

Hospice Caregiver Discipline	LPN
Hospice caregiver name:	Taneeka
Present/not present:	Not present

• *Interview With*

☒ Patient

• *Follows assignment/plan of care*

☒ Yes

• *Patient's needs are being met*

☒ Yes

• *Patient/family/CG satisfied with care provided*

☒ Yes

• *Treats patient with dignity and respect*

☒ Yes

• *Complies with infection control procedures*

☒ Yes

• *Reports changes in patient's condition to RN*

☒ Yes

• *Punctual: notifies office if late*

☒ Yes

• *Dresses properly*

☒ Yes

• *Interacts well with patient/family/CG*

☒ Yes

• *Provides care in a safe manner*

☒ Yes

• *Arrives on time or calls if late*

☒ Yes

• Competent with assigned tasks

☒ Yes

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes

Describe:	Drug Name & Strength	Symptom Treated	# of Doses	Effectiveness (0-10)
	Norco 10/325	Pain to ble	3	8

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): The patient was instructed on energy conservation techniques, including pacing activities throughout the day to minimize fatigue. He was encouraged to rest as needed and to take naps when feeling overly exerted. Education was also provided on managing morning dizziness by sitting up slowly and allowing time before standing. The patient verbalized understanding and was advised to report any worsening fatigue, dizziness, or other changes in condition to the care team.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Additional Information/Comments: The patient is a 64-year-old Black male with a diagnosis of atherosclerotic heart disease of the native coronary artery without angina, currently receiving hospice services. He was alert, oriented to person, place, and time, and cooperative during today's visit. He continues to require assistance with 3 of 6 activities of daily living and is supported by a paid caregiver. The patient reports persistent fatigue and generalized weakness. He also describes experiencing dizziness upon waking in the mornings, which improves after sitting at the edge of the bed for several minutes. Utilizing Norco 10/325 mg for pain to ble , right shoulder , and lower back . Patient reports a near miss , stating his legs have out and he nearly fell . Declined cane , stated he moved a bit too fast . Patient knowledgeable of hospice availability 24/7 and will contact with additional needs

Date: 12/9/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE COMPREHENSIVE VISIT

NURSE COMPREHENSIVE VISIT

Date/Time in:12/10/2025 11:45:00 AM

Date/Time out:12/10/2025 12:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

• Temperature

Type of temperature taken: Temporal:
Temporal temperature: 98.8

• Respiratory Rate

Respiratory rate (respirations per minute): 20
☒ Room air

• Heart Rate

Type of heart rate: Regular
Radial pulse: 96

• Blood Pressure

☒ Sitting
Systolic blood pressure: 130
Diastolic blood pressure: 84

• Pedal Pulses

☒ Present
☒ Right
☒ Left

Current Mental Status

☒ Alert
☒ Oriented

Current Behavioral Status

☒ Appropriate
☒ Cooperative

Speech Status

☒ Clear
☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 0
New, controlled, or uncontrolled? Controlled

Neurosensory

☒ Dizziness

Nutrition

☒ Patient satisfied with intake
Percent of meals eaten: 50

GI Elimination

Date of last bowel movement: 12/10/2025
Bowel Pattern Regular
Bowel sounds, # of quads: 4
☒ WNL
☒ Abdomen: Soft

GU Elimination

Urine output: Good
Urine color: Yellow
☒ Clear

Respiratory

☒ Dyspnea at rest
☒ Dyspnea with exertion
Oxygen saturation percentage: 94

Mobility

☒ Poor endurance
☒ Weakness/fatigue

RN Supervisory Visit (1)

Hospice Caregiver Discipline Homemaker
Present/not present Not present

• Interview With

☒ Patient

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

- *Patient/family/CG satisfied with care provided*

☒ Yes

- *Treats patient with dignity and respect*

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

- *Assignment sheet/POC reviewed*

☒ Yes

• *Assignment sheet/POC updated*

☒ Yes

Comments: Continue POC

RN Supervisory Visit (2)

Hospice Caregiver Discipline LPN
Hospice caregiver name: Taneeka, LVN
Present/not present: Not present

• *Interview With*

☒ Patient

• *Follows assignment/plan of care*

☒ Yes

• *Patient's needs are being met*

☒ Yes

• *Patient/family/CG satisfied with care provided*

☒ Yes

• *Treats patient with dignity and respect*

☒ Yes

• *Complies with infection control procedures*

☒ Yes

• *Reports changes in patient's condition to RN*

☒ Yes

• *Punctual: notifies office if late*

☒ Yes

• *Dresses properly*

☒ Yes

• *Interacts well with patient/family/CG*

☒ Yes

• *Provides care in a safe manner*

☒ Yes

• *Arrives on time or calls if late*

☒ Yes

• *Competent with assigned tasks*

Yes



Comments: Continue POC

Medication Utilization

Have any PRN medications been used in the last 24 hours? No
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction): Education was provided regarding managing morning dizziness by rising slowly from bed, pausing at the edge of the mattress, and avoiding sudden movements to reduce fall risk. The caregiver was instructed to closely supervise morning transitions and encourage the patient to use support when standing or walking. Information was reinforced on monitoring for worsening dizziness, increased weakness, or changes in alertness, and to notify hospice promptly if symptoms become more frequent or severe. Guidance was also given on pacing activities throughout the day to conserve energy and prevent overexertion. Both the patient and caregiver were reassured of ongoing hospice support for symptom changes or concerns.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition

Additional Information/Comments: The patient is a 64-year-old Black male receiving hospice care for atherosclerotic heart disease. During today's visit, he was alert and oriented x3 and cooperative with the assessment. He continues to require assistance with 3 out of 6 ADLs and receives support from a paid caregiver. The patient reports ongoing generalized weakness and describes experiencing episodes of dizziness each morning, which resolve after he sits at the bedside for a short period. No additional changes or new concerns were noted during the visit. Supportive presence was provided throughout the assessment.

Date: 12/10/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

Date/Time in:	12/14/2025 10:00:00 PM
Date/Time out:	12/14/2025 10:01:00 PM
Purpose of progress note:	IDG Narrative
IDG Narrative	The patient is a 73-year-old African American male on hospice care with a diagnosis of atherosclerotic heart disease. He remained alert, cooperative, and pleasant during today's visit. He continues to experience generalized weakness and reduced stamina, which contribute to his unsteady gait. The patient also reports episodes of intermittent dizziness and requires frequent rest breaks when engaging in activity. Appetite remains poor at approximately 25%, and he describes mild lower back discomfort that is currently well controlled without the need for PRN medication. No acute distress was observed, and vitals were within normal limits. Supportive presence was provided, and the current plan of care continues to be effective and appropriate.

Date: 12/14/2025

Electronically signed by: REGINA PURIFOY RN

Patient: ALVIN SCOTT
Medical Record: #87169

TREATMENT ORDERS

TREATMENT ORDERS

Ordering physician: DIEGO F RESTREPO MD
Order: Admit to Hospice: Dx: - Athscl heart disease of native coronary artery w/o ang pctrs
Patient is an OOH-DNR
Diet & Activity as tolerated
DME: Hospital Bed, Bedside Table, BSC, Wheelchair
Date ordered: 10/28/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

ADMISSION ORDERS / INITIAL PLAN OF CARE

Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.
- ☒ May place Hospice Emergency Medication Kit in patient's residence and use PRN as directed.

Nutrition Orders

- ☒ Oral diet orders
- Describe: Diet as tolerated

Activity Orders

- ☒ As tolerated

Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

INITIAL PLAN OF CARE

Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 1
- +PRN Weekly Visits 1
- Reason for PRN Visits Change in condition

Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment
- ☒ Assist patient/family in planning future care
- ☒ Assist with access to community resources
- ☒ Assist with Advance Directives
- ☒ Assist with funeral planning
- ☒ Assist patient/family in obtaining financial assistance
- ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
- ☒ Evaluation for volunteer services
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Spiritual Visits

- ☒ Spiritual needs assessment
- ☒ Phone call to patient/family's own pastor
- ☒ Patient/family receive visits from their own pastor
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Hospice Aide Visits

- ☒ Assist with personal care/hygiene and ADLs
- ☒ Light housekeeping
- ☒ Aide Assignment Sheet/Plan of Care completed
- Weekly Visit Frequency (to be reassessed at next IDG): 5

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Clinical Director: TRACY KING RN

Registered Nurse: VIRGINIA ALLAN RN

Social Worker: ASHLEY DAVIS LMSW

Spiritual Counselor: VERNON MORRISON

Volunteer Coordinator: VERNON MORRISON

Bereavement Coordinator: VERNON MORRISON

Date: 10/28/2025

Electronically signed by: VIRGINIA ALLAN RN
Electronically signed by: TRACY KING RN
Electronically signed by: VERNON MORRISON
Electronically signed by: ASHLEY DAVIS LMSW
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Anxiety/Agitation

Measurable Goals: Patient and family/caregiver will report understanding of the causes and treatment of anxiety.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Anxiety/Agitation	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind	10/30/2025 7:16 AM	TRACY KING RN
2	Potential for Anxiety/Agitation	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind	11/20/2025 3:06 PM	TRACY KING RN
3	Potential for Anxiety/Agitatio	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind	12/3/2025 10:14 PM	TRACY KING RN
4	Potential for Anxiety/Agitatio	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind	12/17/2025 9:06 AM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Cognitive Loss/Dementia

Measurable Goals: Provide positive/enjoyable experiences for resident that do not involve overly demanding tasks and stress.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Cognitive Loss/Dementia	1	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Review medications to determine interactions and which medications can be discontinued because they are no longer effective.	10/30/2025 7:20 AM	TRACY KING RN
2	Cognitive Loss/Dementia	1	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Review medications to determine interactions and which medications can be discontinued because they are no longer effective	11/20/2025 3:05 PM	TRACY KING RN
3	Cognitive Loss/Dementia	1	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Review medications to determine interactions and which medications can be discontinued because they are no longer effective	12/3/2025 10:13 PM	TRACY KING RN
4	Cognitive Loss/Dementia	1	Assist family to include the resident in discussions and decisions regarding the future to the extent possible. Review medications to determine interactions and which medications can be discontinued because they are no longer effective	12/17/2025 9:05 AM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Constipation
Measurable Goals: Patient will report no discomfort with elimination.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	10/30/2025 7:25 AM	TRACY KING RN
2	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	11/20/2025 3:03 PM	TRACY KING RN
3	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	12/3/2025 10:12 PM	TRACY KING RN
4	Potential for Constipation	2	Assess bowel pattern, bowel sounds, last bowel movement, and symptoms of constipation with every visit. Frequency, Size of stool, Change of consistency, Sense of rectal fullness/pressure, Rectal pain on defecation, Inability to pass stool, Bloating or distended abdomen, Abdominal pain, discomfort, cramping, Low back pain, Straining, Fatigue, malaise, lethargy, Dull headache, Nausea and/or vomiting, Change in amount of gas passed rectally, Oozing of liquid stool, Anorexia	12/17/2025 9:04 AM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Depression
Measurable Goals: Patient's reactive depression will be reduced or relieved.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents	10/30/2025 7:33 AM	TRACY KING RN
2	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents Date: 10/30/2025 7:33 AM	11/20/2025 3:01 PM	TRACY KING RN
3	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents	12/3/2025 10:11 PM	TRACY KING RN
4	Potential for Depression	1	Assess for possible physical causes of depression: Unmanaged pain, Fatigue, weakness, Lack of sleep, Neurological changes, such as increased intracranial pressure or brain metastases, Infection, Medication side effects from steroids or chemotherapy agents	12/17/2025 7:50 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Pain

Measurable Goals: Patient will report that pain is reduced to the level personally desired: _____ on a scale of 1-10

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Pain	2	Instruct patient and family on the causes of pain and changing nature of pain in terminally ill patients. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	10/31/2025 1:00 PM	TRACY KING RN
2	Potential for Pain	2	Instruct patient and family on the causes of pain and changing nature of pain in terminally ill patients. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	11/20/2025 3:00 PM	TRACY KING RN
3	Potential for Pain	2	Instruct patient and family on the causes of pain and changing nature of pain in terminally ill patients. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	12/3/2025 10:11 PM	TRACY KING RN
4	Potential for Pain	1	Instruct patient and family on the causes of pain and changing nature of pain in terminally ill patients. Assess pain rating on a scale of 1-10 with every visit. Notify Case Manager if pain is greater than _____.	12/17/2025 7:49 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Edema/Lymphedema

Measurable Goals: Edema will be reduced or maintained, and complications of edema will be prevented or minimized.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity	10/31/2025 1:01 PM	TRACY KING RN
2	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity	11/20/2025 2:58 PM	TRACY KING RN
3	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity	12/3/2025 10:10 PM	TRACY KING RN
4	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity	12/17/2025 7:47 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Diarrhea

Measurable Goals: Patient and family will report understanding of disease process and management of loose, frequent stools.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Diarrhea	2	Instruct patient to replenish fluids to maintain hydration Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	10/31/2025 1:04 PM	TRACY KING RN
2	Potential for Diarrhea	2	Instruct patient to replenish fluids to maintain hydration Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	11/20/2025 2:57 PM	TRACY KING RN
3	: Potential for Diarrhea	2	Instruct patient to replenish fluids to maintain hydration Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	12/3/2025 10:07 PM	TRACY KING RN
4	Potential for Diarrhea	2	Instruct patient to replenish fluids to maintain hydration Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	12/17/2025 7:46 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Dyspnea
Measurable Goals: Patient will report decreased feelings of breathlessness and increased comfort.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Potential for Dyspnea	2	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy.	10/31/2025 1:06 PM	TRACY KING RN
2	Potential for Dyspnea	2	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy.	11/20/2025 2:56 PM	TRACY KING RN
3	Potential for Dyspnea	2	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy.	12/3/2025 10:06 PM	TRACY KING RN
4	Potential for Dyspnea	1	Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites If patient reports improved symptoms with use of oxygen, continue oxygen therapy.	12/17/2025 7:44 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

CARE PLAN

Problem being care planned: Bereavement
Measurable Goals: Family members will share grief with each other.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Patient lives in a facility who has rejected Chaplain support for its residence. Chaplain will reassess at time and death.	0	Assess family's grief response.	10/31/2025 8:56 PM	DAMIEN DAWSON M.DIV
2	Patient lives in a facility who has rejected Chaplain support for its residence. Chaplain will reassess at time and death.	0	Assess family's grief response.	11/20/2025 2:56 PM	TRACY KING RN
3	Patient lives in a facility who has rejected Chaplain support for its residence. Chaplain will reassess at time and death.	0	Assess family's grief response.	12/3/2025 10:06 PM	TRACY KING RN
4	Patient lives in a facility who has rejected Chaplain support for its residence. Chaplain will reassess at time and death	1	Assess family's grief response.	12/17/2025 7:43 PM	TRACY KING RN

Patient: ALVIN SCOTT
Medical Record: #87169

IDG UPDATE

From date: 11/6/2025
To date: 11/20/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	No medications									
	Comfort Kit Medications:									
	10/28/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
	10/28/2025		Atropine sulfate oral Soluntion	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
	10/28/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active

10/28/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/28/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/28/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/28/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/28/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 11/6/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/30/2025	10/30/2025
Cognitive Loss/Dementia	10/30/2025	10/30/2025
Constipation	10/30/2025	10/30/2025
Depression	10/30/2025	10/30/2025
Pain	10/31/2025	10/31/2025
Edema/Lymphedema	10/31/2025	10/31/2025
Diarrhea	10/31/2025	10/31/2025
Dyspnea	10/31/2025	10/31/2025
Bereavement	10/31/2025	10/31/2025

Care Plan list reflects care plan orders as of 11/6/2025

Treatment Order List:

Name	Start Date	DC Date
No Physician Orders		

Physician orders list reflects physician orders as of 11/6/2025

IDG Narrative:

The patient, a 64-year-old single, African-American male, has been admitted under routine home care hospice services with a primary diagnosis of atherosclerotic heart disease of native coronary artery without angina pectoris (I25.10). His admission from a community residential setting occurred on October 28, 2025. He is classified as Disaster Code 2 and holds a "Do Not Resuscitate" (DNR) status of No.

The patient's comorbid conditions include essential hypertension (I10), unspecified atherosclerosis (I70.90), benign prostatic hyperplasia with lower urinary tract symptoms (N40.1), weakness (R53.1), unspecified pain (R52), repeated falls (R29.6), unspecified anxiety disorder (F41.9), unspecified constipation (K59.00), a personal history of urinary tract infections (Z87.440), and chronic diastolic congestive heart failure (I50.32). Additionally, unrelated conditions include type 2 diabetes mellitus with diabetic chronic kidney disease (E11.22) and unspecified hyperlipidemia (E78.5).

The patient, who is a Christian and speaks English as his primary language, has an allergy to Vancomycin and does not currently have arrangements with a funeral home. He refuses supplemental oxygen and is no longer pursuing aggressive medical treatment. He voluntarily concurs with the current hospice plan of care.

Since admission, the patient's general condition reflects a PPS score of 50 and an NYHA classification of stage 4. He has exhibited progressive symptoms, including increased shortness of breath upon ambulation and noticeable bilateral edema, which is being managed with Lasix and compression socks. Over the past six months, there has been a weight decline from 180 to 150 pounds, resulting in a Body Mass Index (BMI) of 22. The patient now consumes only 50% of a standard adult meal due to a reduced appetite.

The patient is clinically assessed as a 3/6, indicating a moderate level of functional impairment. His care plan focuses on symptom management and maintaining quality of life within the limitations of his current health status.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: REGINA PURIFOY RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor: DAMIEN DAWSON M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 11/6/2025

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: VERNON MORRISON
Electronically signed by: DAMIEN DAWSON M.DIV
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

IDG UPDATE

From date: 11/20/2025
To date: 12/4/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update: All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/28/2025		24 HR glipizide 10 MG Extended Release Oral Tablet	10 mg	1tablet Extended Release Oral Tablet	Oral Pill	Once daily	DM	No	Active
	10/28/2025		acetaminophen 325 MG / hydrocodone bitartrate 10 MG Oral Tablet [Norco]	325-10 mg	1 tablet Oral Tablet	Oral Pill	Four times daily	Pain	Yes	Active
	10/28/2025		esomeprazole	20 mg	1 tablet	po	Once daily	GERD	Yes	Active

		20 MG Delayed Release Oral Capsule [Nexium]		Delayed Release Oral Capsule					
10/28/2025		furosemide 20 MG Oral Tablet	20 mg	20mg Oral Tablet	po	Once daily	diuretics	Yes	Active
11/14/2025		melatonin 3 MG Oral Tablet	3 mg	1 tablet Oral Tablet	po	Give once at bedtime	insomnia	Yes	Active
11/14/2025		Promethazine	6.25/5ml	5 ml	Po	Every 4-6 hours prn	Cough / allergies	Yes	Active
11/14/2025	11/24/2025	amoxicillin 875 MG / clavulanate 125 MG Oral Tablet [Augmentin]	875-125 mg	1tablet Oral Tablet	Oral Pill	Twice daily for10 days	URI	Yes	Active
Comfort Kit Medications:									
10/28/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/28/2025		Atropine sulfate oral Soluntion	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/28/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/28/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/28/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/28/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/28/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/28/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 11/20/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/30/2025	11/20/2025
Cognitive Loss/Dementia	10/30/2025	11/20/2025
Constipation	10/30/2025	11/20/2025
Depression	10/30/2025	11/20/2025
Pain	10/31/2025	11/20/2025
Edema/Lymphedema	10/31/2025	11/20/2025
Diarrhea	10/31/2025	11/20/2025
Dyspnea	10/31/2025	11/20/2025
Bereavement	10/31/2025	11/20/2025

Care Plan list reflects care plan orders as of 11/20/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: - Athscl heart disease of native coronary artery w/o ang pctrs Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/28/2025	

Physician orders list reflects physician orders as of 11/20/2025

IDG Narrative: Ongoing treatment for uri initiated 11/14/25: Augmentin 875-125 mg bid x 10 days Continue to meet spiritual needs of patient through socialization, scripture and prayer

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: REGINA PURIFOY RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor BRETT BRATCHER M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 11/20/2025

Electronically signed by: REGINA PURIFOY RN
Electronically signed by: VERNON MORRISON
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: BRETT BRATCHER M.DIV
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

IDG UPDATE

From date: 12/4/2025
To date: 12/18/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/28/2025		24 HR glipizide 10 MG Extended Release Oral Tablet	10 mg	1tablet Extended Release Oral Tablet	Oral Pill	Once daily	DM	No	Active
	10/28/2025		acetaminophen 325 MG / hydrocodone bitartrate 10 MG Oral Tablet [Norco]	325-10 mg	1 tablet Oral Tablet	Oral Pill	Four times daily	Pain	Yes	Active
	10/28/2025		esomeprazole 20 MG Delayed	20 mg	1 tablet Delayed Release	po	Once daily	GERD	Yes	Active

		Release Oral Capsule [Nexium]		Oral Capsule					
10/28/2025		furosemide 20 MG Oral Tablet	20 mg	20mg Oral Tablet	po	Once daily	diuretics	Yes	Active
11/14/2025		melatonin 3 MG Oral Tablet	3 mg	1 tablet Oral Tablet	po	Give once at bedtime	insomnia	Yes	Active
11/14/2025		Promethazine	6.25/5ml	5 ml	Po	Every 4-6 hours prn	Cough / allergies	Yes	Active
Comfort Kit Medications:									
10/28/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/28/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/28/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/28/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/28/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/28/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/28/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/28/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/4/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/30/2025	12/3/2025
Cognitive Loss/Dementia	10/30/2025	12/3/2025
Constipation	10/30/2025	12/3/2025
Depression	10/30/2025	12/3/2025
Pain	10/31/2025	12/3/2025
Edema/Lymphedema	10/31/2025	12/3/2025
Diarrhea	10/31/2025	12/3/2025
Dyspnea	10/31/2025	12/3/2025
Bereavement	10/31/2025	12/3/2025

Care Plan list reflects care plan orders as of 12/4/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: - Athscl heart disease of native coronary artery w/o ang pctrs Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/28/2025	

Physician orders list reflects physician orders as of 12/4/2025

IDG Narrative:

Continue plan of care Chaplain/ Bereavement support offered and declined by patient/family/caregiver

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: REGINA PURIFOY RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor: BRETT BRATCHER M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 12/4/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: REGINA PURIFOY RN
Electronically signed by: BRETT BRATCHER M.DIV
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

IDG UPDATE

From date: 12/18/2025
To date: 1/1/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:									
	10/28/2025		24 HR glipizide 10 MG Extended Release Oral Tablet	10 mg	1tablet Extended Release Oral Tablet	Oral Pill	Once daily	DM	No	Active
	10/28/2025		acetaminophen 325 MG / hydrocodone bitartrate 10 MG Oral Tablet [Norco]	325-10 mg	1 tablet Oral Tablet	Oral Pill	Four times daily	Pain	Yes	Active
	10/28/2025		esomeprazole 20 MG Delayed	20 mg	1 tablet Delayed Release	po	Once daily	GERD	Yes	Active

		Release Oral Capsule [Nexium]		Oral Capsule					
10/28/2025		furosemide 20 MG Oral Tablet	20 mg	20mg Oral Tablet	po	Once daily	diuretics	Yes	Active
11/14/2025		melatonin 3 MG Oral Tablet	3 mg	1 tablet Oral Tablet	po	Give once at bedtime	insomnia	Yes	Active
11/14/2025		Promethazine	6.25/5ml	5 ml	Po	Every 4-6 hours prn	Cough / allergies	Yes	Active
Comfort Kit Medications:									
10/28/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
10/28/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
10/28/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
10/28/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
10/28/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
10/28/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
10/28/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
10/28/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 12/18/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	10/30/2025	12/3/2025
Cognitive Loss/Dementia	10/30/2025	12/3/2025
Constipation	10/30/2025	12/3/2025
Depression	10/30/2025	12/17/2025
Pain	10/31/2025	12/17/2025
Edema/Lymphedema	10/31/2025	12/17/2025
Diarrhea	10/31/2025	12/17/2025
Dyspnea	10/31/2025	12/17/2025
Bereavement	10/31/2025	12/17/2025

Care Plan list reflects care plan orders as of 12/18/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: - Athscl heart disease of native coronary artery w/o ang pctrs Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	10/28/2025	

Physician orders list reflects physician orders as of 12/18/2025

IDG Narrative:

The patient is a 73-year-old African American male on hospice care with a diagnosis of atherosclerotic heart disease. He remained alert, cooperative, and pleasant during today's visit. He continues to experience generalized weakness and reduced stamina, which contribute to his unsteady gait. The patient also reports episodes of intermittent dizziness and requires frequent rest breaks when engaging in activity. Appetite remains poor at approximately 25%, and he describes mild lower back discomfort that is currently well controlled without the need for PRN medication. No acute distress was observed, and vitals were within normal limits. Supportive presence was provided, and the current plan of care continues to be effective and appropriate. Chaplain / Bereavment services offered and declined by patient/ facility/ caregiver

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Registered Nurse:	ANGELA WILLIAMS RN
Social Worker:	ASHLEY DAVIS LMSW
Spiritual Counselor	BRETT BRATCHER M.DIV
Volunteer Coordinator:	VERNON MORRISON
Bereavement Coordinator:	VERNON MORRISON

Date: 12/18/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: BRETT BRATCHER M.DIV
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:	11/4/2025 10:44:00 AM
Date/Time out:	11/4/2025 10:59:00 AM
Purpose of progress note:	Care coordination
Narrative:	Chaplain conducted follow-up outreach by phone to confirm patient’s preference regarding chaplain services. Patient remains consistent in declining spiritual or emotional support, stating that he is “fine” and not interested in visits. No indicators of spiritual distress or emotional crisis noted. Chaplain reiterated ongoing availability and encouraged patient to request support if future needs arise.

Date: 11/5/2025

Electronically signed by: DAMIEN DAWSON M.DIV

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:

11/19/2025 12:26:00 PM

Date/Time out:

11/19/2025 12:27:00 PM

Purpose of progress note:

IDG Narrative

IDG Narrative

Continue to meet spiritual needs of patient through socialization, scripture and prayer

Date: 11/19/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:	11/30/2025 1:35:00 PM
Date/Time out:	11/30/2025 1:45:00 PM
Purpose of progress note:	Phone Calls(s) - Patient Care
Narrative:	Chaplain calls to confirm patient and facility still decline routine chaplain visits. Chaplain remains available PRN should the facility or patient request spiritual or emotional support in the future. Chaplain to maintain PRN status. Documented restrictions in IDG note and continue attempts to engage facility leadership regarding access.

Date: 11/30/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:

12/1/2025 6:20:00 PM

Date/Time out:

12/1/2025 6:21:00 PM

Purpose of progress note:

IDG Narrative

IDG Narrative

Chaplain/ Bereavement support offered and declined by patient/family/caregiver

Date: 12/1/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: ALVIN SCOTT
Medical Record: #87169

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in:

12/15/2025 7:00:00 PM

Date/Time out:

12/15/2025 7:01:00 PM

Purpose of progress note:

IDG Narrative

IDG Narrative

Chaplain / Bereavment services offered and declined by patient/ facility/ caregiver

Date: 12/15/2025

Electronically signed by: BRETT BRATCHER M.DIV